

Medical Administration
Order No. 16 of 0331 and
the first revised Medical Administration Order
of 0526 on March 31, 2016
and the final revised Medical Administration
Order of 0615 No. 21 and June 15 of 5

To the Governors of Each Prefecture

Director-General of the Medical Affairs Bureau of the Ministry
of Health, Labor and Welfare (official seal omitted)

About Medical Plans

Regarding the medical care delivery system in Japan, up until now,
In order to ensure a high-quality and appropriate medical care system,
Promotion of differentiation and cooperation of hospital bed functions and the establishment of a comprehensive community care
system (the comprehensive community care system stipulated in Article 2, Paragraph 1 of the Law Concerning the Development
of Related Laws to Promote Comprehensive Assurance of Medical Care and Nursing Care in the Community (Law No. 83 of 2014)
(The same applies below.)

In addition, the Act amending the Medical Care Act and the Medical Practitioners Act (Act No. 79 of 2018)
In order to improve patients' access to medical care and reduce the workload of doctors, the government will
work to eliminate uneven distribution of doctors through the formulation of a doctor recruitment plan, etc.,
and will impose upper limits on overtime work for doctors in April 2024 under the Act on Partial Revision of
the Medical Care Act, etc. to Provide High-Quality and Appropriate Medical Care Efficiently (Act No. 49 of 2021).

In preparation for the start of the application of the new law, a system will be created to reduce doctors' working hours and maintain their health.
From the perspective of medical professionals, such as reviewing the scope of work of medical-related professions,
We have been working to ensure a high-quality and appropriate medical care system in the region.
It is.

Due to the recent spread of COVID-19, the medical care providers in Japan
This has had a major impact on the medical system, highlighting various issues in local medical care, including emergency medical care.

This resulted in a renewed recognition of the importance of differentiating, strengthening and coordinating medical functions across inpatient, outpatient and home care in the region, as well as the importance of providing necessary medical care under an appropriate division of roles with a view to regional medical care as a whole.

Furthermore, the elderly population will increase rapidly through 2025, and then decline toward 2020.

While the increase in the working-age population will slow down in the future, the productive population, which has already started to decline, will continue to decline from 2025 onwards. With this in mind, it will be necessary to respond to the upper limit on doctors' overtime and holiday work that will come into effect in 2024, and securing manpower to support the regional medical care system will become an increasingly important issue. In light of this, in order to maintain the necessary medical care system in response to the changes in quality and quantity of medical needs due to population decline and aging, it is necessary to steadily advance efforts on the regional medical plan and to work in a unified manner to secure medical personnel. Furthermore, from the perspective of improving the quality and efficiency of medical care, it is also important to promote the use of information and communication technology (ICT) and the digitalization of the medical field. In formulating this medical care plan, the 8th National Health Insurance Council, which was held from June 2021,

Based on the summary of opinions at the review committee on medical plans, etc.,

• Establishment of an integrated medical care system that covers the acute, recovery and chronic stages. • Establishment of a medical care system that crosses disease and business boundaries. • Establishment of a policy cycle through the review of indicators related to the five diseases, six businesses and home medical care.

Strengthening the system

• Ensuring consistency with other plans, such as the long-term care insurance business (support) plan

From these perspectives, the Basic Policy for Ensuring a System for Providing Medical Care (Heisei 19, No. 205) was established pursuant to the provisions of Article 30-3, Paragraph 1 of the Medical Care Act (Heisei 19, No. 205).

The Ministry of Health, Labour and Welfare will revise the Basic Policy (hereinafter referred to as the "Basic Policy") (Ministry of Health, Labour and Welfare Notification No. 70 of 2007).

In addition, the "Guidelines for Drafting Medical Plans" (hereinafter referred to as the "Guidelines") were revised.

Prefectural governments are required to build a system for efficiently providing high-quality and appropriate patient-centered medical care and to ensure the public's peace of mind and trust in medical care by reviewing medical care plans based on the purpose of the revision of the basic policy, thereby making medical care plans function more effectively and promoting efforts to achieve regional medical care plans.

It is also important to continue to

In addition, efficient and high-quality medical care will be provided by promoting differentiation and collaboration of hospital bed functions.

In order to ensure that the establishment of a comprehensive community care system, including the establishment of a system and the enhancement of medical care in homes, etc. (hereinafter referred to as "in-home medical care") and nursing care, is carried out in an integrated manner, it is necessary to ensure consistency between medical plans, prefectural nursing care insurance business support plans (hereinafter referred to as "prefectural nursing care insurance business support plans") prescribed in Article 118, paragraph 1 of the Long-Term Care Insurance Act (Act No. 123 of 1997), and municipal nursing care insurance business plans (hereinafter referred to as "municipal nursing care insurance business plans") prescribed in Article 117, paragraph 1 of the same Act. In addition, when formulating medical plans, with regard to medical care when an infectious disease that may have a significant impact on the lives and health of the public due to its spread is spreading or is likely to do so (hereinafter referred to as "medical care when an emerging infectious disease occurs or spreads"), it is necessary to ensure consistency between the medical care plan and the prevention plan (hereinafter referred to as the "prevention plan") stipulated in Article 10, Paragraph 1 of the Infectious Disease Prevention and Medical Care for Patients Act (Act No. 114 of 1998; hereinafter referred to as the "Infectious Diseases Act") after amendment pursuant to Article 3 of the Act to partially amend the Infectious Disease Prevention and Medical Care for Patients Act, etc. (Act No. 96 of 2022) and the prefectural action plan (hereinafter referred to as the "prefectural action plan") stipulated in Article 7, Paragraph 1 of the Special Measures Act for Pandemic Influenza, etc. (Act No. 31 of 2012), so that emerging infectious diseases can be responded to in a coordinated manner according to the actual conditions of the region.

In light of the above, prefectures are requested to make every effort to thoroughly publicize the purpose and contents of the review, and to ensure that medical plans are created and promoted without any problems, referring to the points to note and the specific procedures outlined in the guidelines below.

Please note that this notification is technical advice based on Article 30-8 of the Law. In addition, "Regarding Medical Plans" (Notice of the Director-General of the Medical Affairs Bureau, No. 0331-57, dated March 31, 2017) is abolished.

remember

1. Creating a medical plan

In preparing medical plans, in accordance with the basic policy, the guidelines and the attached document "Regarding medical systems related to diseases, projects, and home medical care" (Notification of the Director of the Regional Medical Planning Division, Medical Affairs Bureau, Ministry of Health, Labor, and Welfare, No. 0331-14, dated March 31, 2023)

The guidelines for the establishment of a medical system for medical services and home medical care (hereinafter referred to as the "Disease, Business and Home Medical Care Guidelines") shall be used as reference, and the opinions of relevant parties shall be fully taken into consideration in accordance with the current state of the medical care delivery system and the actual conditions of the region, including future changes in medical care demand.

To do something after reading this.

2. Medical Cooperation System (1)

Medical Cooperation System (Ensuring the division of functions and cooperation between medical facilities)

The matters related to the "System for Protecting Health and Welfare" (hereinafter the same) shall be stipulated for five diseases (hereinafter referred to as the "Five Diseases"): cancer, stroke, cardiovascular diseases such as myocardial infarction, diabetes, and mental disorders, as well as six projects (hereinafter referred to as the "Six Projects"): emergency medical care, medical care during disasters, medical care during the outbreak and spread of emerging infectious diseases, medical care in remote areas, perinatal medical care, and pediatric medical care (including pediatric emergency

medical care), and home medical care. In doing so, the evaluation and revision of policies and projects shall be carried out not only from the perspective of the results (output) of the policies and projects, but also from the perspective of the impact they had on outcomes such as the health status of residents and the condition of patients. It is important to improve the situation and strengthen the policy cycle (PDCA cycle, etc.). For this reason, it is necessary to grasp the current situation by using indicators, etc., and then extract issues in terms of measures based on each of the aims of the five diseases, six programs, and home medical care described in the disease/program and home medical care guidelines, set policies and numerical targets to resolve them, and evaluate the progress of these measures.

To do so.

As for chronic obstructive pulmonary disease (COPD), chronic kidney disease (CKD), locomotive syndrome, frailty, pneumonia, femoral neck fractures, etc., although they are not included in the five diseases, it is necessary to take measures in harmony with related policies such as health promotion policies.

(2) In the midst of a declining population and aging society, changes in disease structure and the stability of regional medical care

In order to address issues such as the need for medical care,

In addition, with the cooperation of local medical professionals, etc.,

By sharing and coordinating responsibilities, we can build a system for providing seamless medical care.

In order to respond to the diverse circumstances surrounding medical care and nursing care in each region, it is necessary to utilize limited local social resources efficiently and effectively and to provide local

Cooperation between medical institutions and related organizations is also important in building a community-based comprehensive care system.

3. Securing doctors and medical personnel (excluding doctors)

- (1) With regard to securing medical doctors as specified in Article 30-4, Paragraph 2, Item 11 of the Law and securing medical personnel (excluding medical doctors) as specified in Item 12 of the same Paragraph, it is necessary to proceed with training medical personnel such as medical doctors, dentists, pharmacists, and nurses while forecasting future supply and demand trends, to secure an appropriate supply, and to proceed with addressing uneven distribution among regions and medical departments, etc.

In this regard, efforts to build a system of cooperation between medical facilities will itself be a measure to eliminate uneven distribution, and prefectures will be required to take the lead in reconstructing the system for dispatching doctors to local medical institutions.

Based on this, prefectures are required to hold discussions to determine necessary measures for securing doctors and medical personnel, utilizing local medical care countermeasure councils, etc., in accordance with the provisions of Article 30-23, Paragraph 1 of the Law, and to include the measures determined therein in medical care plans, as well as to make them public and implement them.

To be.

In particular, in order to secure doctors at local medical institutions, it is necessary to work closely with universities and other related institutions to alleviate the concerns of doctors working at local medical institutions regarding their career development, and to take measures to support the securing of doctors at local medical institutions in conjunction with supporting the career development of doctors. Prefectural governments are required to promote necessary measures by utilizing local medical support centers, etc., in consultation with local medical professionals, etc.

In addition, when restrictions on overtime and holiday work for doctors are imposed from April 2024, it is important to support each medical institution in taking measures to improve the working environment so that doctors can work while maintaining their own health, such as reducing the workload of doctors by promoting task shifts/shares, including securing medical administrative assistants.

- (2) Regarding ensuring medical safety under Article 30-4, Paragraph 2, Item 13 of the Law, in relation to matters related to the safety management of medical equipment, the arrangement and operation status, etc. of advanced medical equipment shall be confirmed and an evaluation, including maintenance and inspection, shall be conducted.

In addition, for clinics that have medical equipment such as CT and MRI, the warranty for said equipment is required.

Require regular reports on the status of medical safety initiatives, including inspections.

and.

- (3) In relation to the objectives of the development of regional medical support hospitals under Article 30-4, paragraph 3, item 1 of the Act and the objectives of the development of medical facilities taking into account other functions of medical facilities, when the functions to be performed by a specific hospital, etc. are to be included in the medical plan, sufficient coordination of opinions shall be conducted in advance with the operator of the hospital.

4. Standard number of hospital beds and special provisions for specific hospital beds, etc.

- (1) The calculation of the standard number of hospital beds prescribed in Article 30-4, Paragraph 2, Item 17 of the Law is made for hospital beds and clinic beds (hereinafter referred to as "hospital beds, etc.>"). That is.

The standard number of hospital beds and the establishment of secondary and tertiary medical care areas (meaning the areas prescribed in items 14 and 15 of the same paragraph) shall be implemented in accordance with the standards prescribed by the Ministry of Health, Labor and Welfare ordinance. This is because, in order to ensure the appropriate allocation of hospital beds, etc., it is difficult to expect effectiveness unless it is implemented uniformly in all prefectures. Because it is.

- (2) Existing secondary medical care areas will be integrated into one area providing medical care related to hospitalization.

In cases where the population size of a region has not yet been established, the region should consider revising the system. In doing so, the region should consider the actual situation of the population size, the inflow of patients into the region, and the outflow of patients from the region, because the population size of the region has a significant impact on the trends in patients receiving medical care.

In particular, existing secondary medical care areas with a population size of less than 200,000, where the estimated inflow rate of hospitalized patients (hereinafter referred to as the "inflow rate") of medical care beds and general hospital beds in hospitals within the secondary medical care area is less than 20% and the estimated outflow rate of hospitalized patients (hereinafter referred to as the "outflow rate") is 20% or more, are not considered to be integrated areas providing medical care related to hospitalization. Therefore, it is necessary to consider revising the settings.

If the result of the review is that no review is to be carried out, the reasons (geographical conditions, area of the area, geographical access, etc.) must be clearly stated. Furthermore, for secondary medical care areas with a population of 1 million or more, the area must be designated as a concept area (Article 30-4 of the Law). The plan area as provided for in Paragraph 2, Item 7 (hereinafter the same).

In light of the fact that many issues arise, the establishment of areas may be revised as necessary.

In addition, the Regional Medical Plan Coordination Conference (meaning the forum for discussion stipulated in Article 30-14, Paragraph 1 of the Law) should be held separately for each area that is further divided within the plan area and for each topic of discussion, such as regional medical issues.

We will make operational improvements such as:

In addition, if the planned area and the secondary medical care area are different, it is appropriate to make them consistent. Therefore, it is necessary to align the secondary medical care area with the planned area.

To make corrections.

- (3) Exceptions under Article 30-4, paragraphs 9 and 10 of the Act shall be made in cases where a rapid increase in population is expected due to large-scale urban development, etc., making it inappropriate to calculate the number of hospital beds based on the current population, where an announcement has been made regarding the outbreak of infectious diseases such as novel influenza as provided for in Article 16, paragraph 2 of the Infectious Diseases Act, or where there is an abnormally large number of people suffering from a particular disease, and where it is recognized that there is a special demand for hospital beds.

- (4) Subject to the special provisions for specific hospital beds pursuant to Article 30-4, Paragraph 11 of the Law Hospital beds, etc., that are particularly necessary in each area in the future, can be exceptionally developed as necessary even if there are more than the standard number of hospital beds in each area. In this case, the number of beds subject to the exception will be the minimum necessary,

taking into account the number of existing hospital beds with the same function and characteristics as the beds in the application.

thing.

In addition, the hospital beds that are subject to these special provisions will be calculated as the number of existing hospital beds.

- (5) With regard to the permission under the provisions of Article 30-4, Paragraphs 10 and 11 of the Law, (3)

In cases where it is deemed that the circumstances set forth in (4) above no longer exist, or where the business related to the functions of the special beds in (4) is no longer being carried out, the number of beds related to the special permission may be reduced to the extent that it exceeds the standard number of beds.

What you can do.

- (6) The special provisions under Article 30-4, Paragraph 12 of the Law shall not apply to regional medical cooperation promotion corporations (hereinafter the same) of a participating corporation (hereinafter the participating corporation as defined in Article 70, Paragraph 1 of the Law; the same shall apply hereinafter)

Necessary to promote the achievement of the regional medical vision among each other or within the same participating corporation

The number of hospital beds is sufficient, the total number of hospital beds is not increasing, and the opinion of the Regional Medical Cooperation Promotion Council (meaning the Regional Medical Cooperation Promotion Council prescribed in Article 70-3, Paragraph 1, Item 16 of the Law) of the Regional Medical Cooperation Promotion Corporation is heard and the proposal can be implemented only if the proposal is implemented while respecting said opinion. In addition, when

approving the necessary number of hospital beds, the proposal must be approved in accordance with the direction of discussions at the Regional Medical Cooperation Plan Coordination Meeting in the planning area related to the application for the increase in hospital beds, etc. Verify that the information is correct.

(7) Regarding the exceptions under Article 30-4, paragraphs 9 to 12 of the Law, the prefectural medical council shall be consulted. In doing so, the reasons why the exceptions are necessary and the basis for calculating the number of hospital beds to be treated as exceptions shall be clearly stated, and the opinion of the prefectural medical council shall be heard.

In addition, when consulting with the Minister of Health, Labour and Welfare regarding the number of beds to be treated as an exception based on the above provisions (excluding the provisions of Article 30-4, Paragraph 12 of the Law), the opinion of the Prefectural Medical Council must be attached to the application form (Appendix Forms 1 and 2) that describes the reasons for the need for exception treatment and the basis for calculating the number of hospital beds to be treated as an exception.

(8) When prefectures develop convalescent and general hospital beds, they should consider not only the relationship between existing hospital beds and the standard number of hospital beds, but also the future number of hospital beds required in the regional medical plan, so that they are consistent with the efforts to achieve the regional medical plan. Specifically, "Matters that prefectures should take note of when developing hospital beds based on the regional medical plan" (Notice of the Director of the Regional Medical Planning Division, Medical Affairs Bureau, Ministry of Health, Labor and Welfare, dated June 23, 2017, No. 0623-1) Please refer to the notes in etc.

5. Number of existing hospital beds and number of hospital beds requested (1)

Enforcement Regulations of the Medical Care Act (Ministry of Health and Welfare Ordinance No. 50 of 1948; hereinafter referred to as the "Regulations")

In accordance with Article 30-33, Paragraph 1, Item 1, the hospitals or clinics established by the government and under the jurisdiction of the Imperial Household Agency, the Ministry of Defense, etc., and hospitals or clinics that only treat employees of specific offices or business establishments and their families, etc., are adjusted based on the proportion of users of the hospital beds, such as non-members other than personnel (corps members) and their families, and non-employees and their families, in the calculation of the number of existing hospital beds and the number of hospital beds related to the application.

This is because, with regard to hospital or clinic beds, except for those used by outsiders, they cannot be said to be providing medical care to the general public.

When an application for permission to open a hospital or clinic is submitted, the purpose of the establishment will be thoroughly examined. The same shall apply when an application for a change in the purpose of the establishment is submitted.

(2) With regard to beds in radiation therapy rooms (excluding special measures rooms as provided for in Article 30-12, paragraph 2 of the Ordinance), these are beds used exclusively for radiation therapy, and therefore shall not be counted as the number of existing beds or the number of beds related to the application.

(3) Beds in hospitals that are national and non-national sanatoriums for Hansen's disease shall not be included in the existing number of beds. (4) Beds in hospitals that are designated inpatient medical institutions designated by the Minister of Health, Labour and Welfare pursuant to the provisions of Article 16, paragraph 1 of the Law on Medical Treatment and Observation, etc. of Persons Who Have Committed Serious Harm to Others in a State of Mental Incapacity, etc. (Law No. 110 of 2003) (limited to those related to medical treatment through hospitalization under Article 42, paragraph 1, item 1 or Article 61, paragraph 1, item 1 of the same Law) shall be included in the existing number of beds.

Don't do that.

(5) Among the general beds of a clinic, the "specified beds" as defined in Article 3, Paragraph 3 of the Supplementary Provisions of the Law Partially Amending the Medical Care Act, etc. to Establish a System for Providing Quality Medical Care (Law No. 84 of 2006) shall not be included in the number of existing beds until a date to be determined by a separate Cabinet Order. (6) With regard to the medical care beds or general beds of a clinic, when the clinic has passed a deliberation at the Prefectural Medical Council as a clinic that falls under Article 1-14, Paragraph 7, Items 1 or 2 of the Ordinance, the beds shall be established by notification instead of by application for permission to the prefectural governor, but the notified beds shall be included in the calculation of the number of existing beds.

6. Procedures for creating medical plans

(1) The "policies closely related to medical care" in Article 30-4, Paragraph 14 of the Law refers to the basic The policy etc. set forth in Policy No. 11 applies.

(2) Pursuant to the provision of Article 30-4, Paragraph 15 of the Law, when formulating a medical plan, prefectures shall take into consideration the actual situation of medical supply and demand in areas surrounding prefectural borders.

When necessary, liaise and coordinate with the prefectures concerned.

This includes the establishment of a medical cooperation system for five diseases, six projects, and home medical care.

Depending on the content of the policy, a wider area response may be required. Therefore, prefectures are required to cooperate with neighboring prefectures, etc. as necessary, in order to ensure medical care in areas around the prefecture borders, rather than focusing on self-containment within the prefecture.

- (3) The "organization of persons with academic or expert experience related to medical treatment or dispensing" referred to in Article 30-4, paragraph 16 of the Law may include medical associations, dental associations, and pharmaceutical associations established on a prefectural level.
- (4) Pursuant to the provision of Article 30-4, paragraph 17 of the Law, opinions of the insurer council (meaning the council provided for in Article 157-2, paragraph 1 of the Law on Assurance of Medical Care for Elderly People (Law No. 80 of 1982); the same applies hereinafter) established in each prefecture shall be heard in advance regarding the preparation of medical care plans, etc.
- (5) The change in the medical plan under the provision of Article 30-4, paragraph 18 of the Law means a change based on the provision of Article 30-6 of the Law. For example, in the medical cooperation system for each of the five diseases, six services, and home medical care, when changing the medical care facility that provides the medical care function, the change in the medical plan based on this provision is
Not hitting.
- (6) Pursuant to the provisions of Article 30-6 of the Law, medical plans shall be surveyed, analyzed and evaluated every six years, and revised if necessary. In addition, matters relating to home medical care, securing of doctors and outpatient medical care shall be surveyed, analyzed and evaluated every three years, and revised if necessary.
- (7) In order to create medical plans, prefectural long-term care insurance business support plans, and municipal long-term care insurance business plans in an integrated manner and ensure consistency among these plans, a forum for consultation as stipulated in Article 2-2-1 of the Basic Policy for Comprehensively Ensuring Medical Care and Long-Term Care in the Region (Ministry of Health, Labour and Welfare Notification No. 354 of 2014) shall be established. In addition, with regard to medical care, particularly in the event of the outbreak or spread of an emerging infectious disease, it is important to ensure consistency with prevention plans and prefectural action plans, and to utilize the cooperation council as stipulated in Article 10-2 of the Infectious Diseases Act as necessary so that local governments can cooperate in responding to infectious diseases according to the actual situation of the region.

Since FY 1994 is the start of the formulation of prevention plans by prefectures and long-term care insurance business plans by municipalities, care should be taken to prepare a system for sharing the formulation schedules of these plans between prefectures and municipalities and advancing discussions.

In addition, since it is important to ensure consistency regarding the new service needs for nursing care facilities, home medical care, etc. that arise from the functional differentiation and collaboration of hospital beds, necessary matters will be discussed at the consultation forum so that the nursing care development goals set out in the prefecture nursing care insurance business support plan and the city/town/village nursing care insurance business plan and the home medical care development goals set out in the medical plan are consistent.

7. Promotion of medical planning

(1) Article 30-7, paragraph 1 of the Law states that the operator and manager of a medical facility shall:

In order to contribute to the promotion of the achievement of medical plans, the hospital or clinic shall endeavor to cooperate as necessary to build a medical cooperation system, and in providing necessary cooperation, the hospital or clinic shall endeavor to cooperate with other medical facilities and fulfill their respective prescribed roles, according to the provision of home medical care or the provision of necessary support in cooperation with welfare services, according to the provision of paragraph 3 of the same article.

That is what is being done.

(2) The opening of hospitals under the provision of Article 30-7, Paragraph 4 of the Law is not limited to the shared use of hospital beds and medical equipment, but is broadly defined to include the holding of case study meetings and training sessions with the participation of local doctors who do not work at the hospital. To be.

In addition, in promoting the medical plan, due consideration must be given so as not to impede medical or dental education or research at the university. Note that the "medical services of the hospital" in the same paragraph includes education or research of the university at the university's affiliated hospital.

(3) Regarding government subsidies based on the provisions of Article 30-9 of the Law,

In order to promote this, the contents of medical plans will be taken into consideration.

(4) In the event that the opening of a hospital, etc. is the subject of a recommendation under Article 30-11 of the Law from the viewpoint of promoting medical care plans, the Welfare and Medical Care Agency

The Company has decided not to provide loans.

8. Recommendations from Prefectural Governors

(1) "When it is particularly necessary to promote the achievement of medical plans" as set forth in Article 30-11 of the Law

In principle, this refers to cases where a person other than those listed in each item of Article 7-2, Paragraph 1 of the Act applies for permission to open a hospital, increase the number of hospital beds in a hospital, or change the type of hospital beds, or applies for permission to establish hospital beds in a clinic or increase the number of hospital beds in a clinic, and, depending on the type of beds, the number of existing hospital beds in the secondary medical area or prefectural area including the location of the hospital or clinic has already reached the standard number of hospital beds for that area as set out in the medical plan, or the opening of the hospital or clinic will cause the standard number of hospital beds to be exceeded.

In addition, "recommendations regarding the opening of a hospital, an increase in the number of hospital beds, or a change in the type of beds, or the establishment of beds in a clinic, or an increase in the number of beds in a clinic" refers to recommendations to discontinue each action or to reduce the number of beds applied for. Prior to making a recommendation, prefectural governors should, to the extent possible, make a recommendation to those who intend to open a hospital or clinic, regarding the opening of a hospital or clinic in another district.

It is desirable to give advice.

(2) The recommendation under Article 30-11 of the Law shall not affect the decision on permission or non-permission under Article 7 of the Law.

This must be done before the deadline for the transfer is reached.

(3) With regard to psychiatric beds, tuberculosis beds and infectious disease beds, the standard number of beds is to be calculated for each prefectural area, but in cases where these beds are unevenly distributed in a part of a prefecture and there are no beds of the type related to the application in the secondary medical care area including the location of the hospital for which an application for opening has been filed and in other secondary medical care areas bordering on said area (including secondary medical care areas established in other prefectures), it is considered appropriate not to make such a recommendation the subject even if the number of hospital beds in the prefectural area has already reached the standard number of hospital beds for the area as set out in the medical care plan. In such cases, the opinion of the Prefectural Medical Council shall be heard.

(4) Even if there is a change in the operator of a hospital or clinic,

If the number of beds of each bed type is not increased, no recommendation shall be made.

- (5) Even in the case of the relocation of a hospital or clinic, no recommendation shall be made if there is no increase before or after the relocation in the number of medical beds and general hospital beds within the secondary medical care area in which the hospital or clinic is located, and no increase in the number of psychiatric beds, tuberculosis beds or infectious disease beds within the prefecture.

In addition, if a clinic with specific beds relocates, the name of the clinic

The number of existing hospital beds in the secondary medical area will increase by the number of the specific beds, but if the number of beds by type of hospital bed does not increase before and after the relocation, no recommendation will be made. This shall not apply.

- (6) In the case of the reorganization of two or more medical institutions and the abolition of one or more medical institutions (excluding cases where the reorganization is carried out beyond the secondary medical area) in order to promote the achievement of the regional medical plan, if the total number of hospital beds does not increase before and after the reorganization, and if the plan for the reorganization is approved by the Minister of Health, Labour and Welfare as provided for in Article 12-2-2 of the Act on Promotion of Comprehensive Assurance of Medical Care and Nursing Care in Regions (Act No. 64 of 1989; hereinafter referred to as the "Comprehensive Assurance of Medical Care and Nursing Care Act") after consultation and agreement at the Regional Medical Plan Coordination Council, no recommendation shall be made. However, in consideration of the fact that the area is in an excess of hospital beds, and in light of the intent of the provisions of Article 7-2, paragraph 3 as applied mutatis mutandis pursuant to the provisions of Article 30-12, paragraph 1 of the Act, if the medical institutions to be reorganized are not performing all or part of the services related to hospital beds, the total number of hospital beds after the reorganization shall be the number of hospital beds that are not performing the services, except in cases where it is deemed unavoidable. In addition, in the case where a medical institution subject to reorganization has beds that were established without permission under Article 7, Paragraph 3 of the Law as a case listed in each item of Article 1-14, Paragraph 7 of the Regulations, or beds that are listed in each item of Article 30-32-2, Paragraph 1 of the Regulations, the medical institution must take appropriate measures in light of the purpose of the beds.

and.

- (7) In the case where a person who operates a hospital closes the hospital and establishes beds for a clinic in the place where the hospital was established, if the number of medical beds and general beds in the secondary medical care area where the clinic is located is not increased, No recommendations shall be made.

- (8) National Government (National Institutes for Quantum and Radiological Science and Technology, Japan Maritime Technical Education Agency, Japan Occupational Health and Safety Agency, National Hospital Organization)

National Cancer Center, National Cerebral and Cardiovascular Center,

In the case of hospitals or clinics established by national or local governments (including the National Center of Neurology and Psychiatry, National Center for Global Health and Medicine, National Center for Child Health and Development, National Center for Geriatrics and Gerontology, and national university corporations; the same applies below), the provisions of Article 30-11 of the Act shall not apply pursuant to the provisions of Article 3 of the Enforcement Order of the Medical Care Act (Cabinet Order No. 326 of 1948) based on Article 6 of the Act.

In addition, when the national government intends to open a hospital, increase the number of beds or change the type of beds in a hospital it has opened, or to set up beds in a clinic, increase the number of beds in a clinic or change the type of beds, the competent minister, etc. is to consult with the Minister of Health, Labour and Welfare in advance regarding the plan, pursuant to the provisions of "Regarding the Handling of Hospitals Established by the National Government Following the Enforcement of the Law Partially Amending the Medical Care Law" (Cabinet decision of March 19, 1964) or Article 7-2, Paragraph 7 of the Law.

In this case, I will promptly notify the governors of the prefectures concerned of the fact and a summary of the plan, and I request that they take prompt action to provide the necessary materials for the review of the plan and their opinions from the perspective of promoting the achievement of the medical plan. (9) When making recommendations to persons who intend to establish hospitals affiliated with medical education institutions, or to establishers or managers of hospitals affiliated with medical education institutions, it is desirable to give them an opportunity to express their opinions from the standpoint of medical or dental education and research at universities.

(10) Regarding the establishment of medical beds or general beds at a clinic, if the clinic has gone through a deliberation at the Prefectural Medical Council as one of the following clinics that fall under Article 1-14, paragraph 7, item 1 or 2 of the Ordinance, the beds will be established by notification instead of by application for permission to the prefectural governor, and therefore the clinic will not be subject to a recommendation.

Regarding the following clinics, "Establishment of a system to provide high-quality medical care"

Regarding the partial enforcement of the Act amending the Medical Care Act etc. for the purpose of

(Notification No. 1227017 of the Director-General of the Medical Affairs Bureau of the Ministry of Health, Labour and Welfare dated December 27, 2018)

Please refer to the notes.

ý Necessary for the promotion of the provision of medical care as set forth in Article 30-7, Paragraph 2, Item 2 of the Law

2) Medical facilities for remote areas, perinatal care, pediatric care, emergency care and other facilities necessary for the establishment of a comprehensive regional care system

Clinics necessary to provide high-quality and appropriate medical care

9. Regulations on the opening of hospitals or clinics of a public nature

The regulations on the establishment of hospitals or clinics of a public nature established by the entities listed in Article 7-2, Paragraph 1 of the Law after the date of public announcement of the medical plan pursuant to the provisions of Article 30-4, Paragraph 18 of the Law shall be based on the areas and standard number of hospital beds set out in the said medical plan. It is something that is carried out as a standard.

(Attachment Form 1)

Number Year Month Day

Minister of Health, Labour and Welfare

temple

Prefectural Governor

Regarding consultations based on the provisions of Article 5-2, Paragraph 2 of the Enforcement Order of the Medical Care Act

Pursuant to the provisions of Article 5-2, Paragraph 2 of the Enforcement Order of the Medical Care Act based on the provisions of Article 30-4, Paragraph 9 of the Medical Care Act

Regarding the increase in the number of hospital beds, we will attach relevant documents and discuss the following:

Attached

documents: 1. Written statement of the opinion of the Prefectural Medical Council 2. Map of the relevant area and other reference documents

1. Number of hospital beds to be added		2. Type of hospital bed to be added	
3. Areas to be added			
4. Reasons for the need for an additional payment			
5. Basis for calculating the number of hospital beds to be added			
6. Current status and plans for related medical facilities			
7 preparations Test			

Notes on filling out the form:

In the "4. Reasons for needing the additional payment" column, if you are planning to develop a specific hospital due to an increase in the incidence of a specific disease, etc., please indicate this fact as well as an outline of the development plan for the hospital and the reasons for selecting the location, etc.

(Attachment Form 2)

Number Year Month Day

Minister of Health, Labour and Welfare

temple

Prefectural Governor

Regarding consultations based on the provisions of Article 5-3, Paragraph 2 (or Article 5-4, Paragraph 2) of the Enforcement Order of the Medical Care Act

Regarding the calculation of the number of hospital beds pursuant to the provisions of Article 5-3, Paragraph 2 (or Article 5-4, Paragraph 2) of the Enforcement Order of the Medical Care Act based on the provisions of Article 30-4, Paragraph 10 (or Paragraph 11) of the Medical Care Act, we will attach the relevant documents and discuss the following as follows.

Attached

documents: 1. Written statement of the opinion of the Prefectural Medical Council 2. Map of the relevant area and other reference documents

1. Hospital beds that should be given special treatment number		2. Exceptional types of hospital beds	
3. Exceptional areas			
4. Reasons for the need for special provisions			
5. Basis for calculating the number of hospital beds to be subject to the exception			
6. Current status and plans for related medical facilities			
7 preparations Test			

Notes on filling out the form

In the "4. Reasons for the need for an exception" section, if you are planning to develop a specific hospital due to the high incidence of a specific disease, etc., please state that fact, as well as an outline of the development plan for the hospital and the reasons for selecting the location, etc.

(Attachment)

Guidelines for creating medical plans

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Introduction

Prefectures are to establish their own medical plans in accordance with the basic policy established by the Minister of Health, Labour and Welfare and in accordance with the actual conditions of their regions, but the Minister of Health, Labour and Welfare may provide necessary advice to prefectures on the methods of creating medical plans and other important technical matters. This guideline is intended as a handbook for prefectures to use as a reference on such matters.

This is what is shown.

Section 1: Purpose of creating medical plans

The medical care system in Japan is an important foundation for ensuring the health of the nation's citizens and allowing them to live their lives with peace of mind. However, as the environment surrounding medical care is changing dramatically due to the aging of the population, advances in medical technology, and changes in public attitudes, there is a need to create an environment in which everyone can receive medical care with peace of mind.

There are.

In particular, amid a declining and aging population and an increasingly diverse and complex social structure, there is a need to build a medical care system that can respond to the five diseases, while also taking into account the increasing number of patients, in order to improve the quality of life.

Furthermore, with regard to the six projects and home medical care, which are important issues in ensuring regional medical care, it is necessary to establish a medical care delivery system that can respond to these issues so that patients and residents can receive medical care with peace of mind.

In order to promote the appropriate differentiation and cooperation of regional medical functions for each of the five diseases, six programs, and home medical care, and to build an efficient and high-quality medical care delivery system in each region where seamless medical care can be received, policy circulation in medical care plans is required.

It will be important to further strengthen mechanisms (such as the PDCA cycle).

Specifically, based on outcomes such as the health status of residents and the condition of patients, the current state of the medical care delivery system will be grasped, the current medical care plan will be evaluated, and issues will be identified based on each of the desired directions (meaning the desired directions for the five diseases, six programs, and home medical care; the same applies below), measures to resolve these issues will be specified, numerical targets will be set, and progress will be evaluated. When evaluating policies and programs, not only the results (output) of the policies and programs will be considered, but also the health status of local residents, the condition of patients, and the condition of the local community.

It is necessary to incorporate into the policy cycle a mechanism (such as the PDCA cycle) for reviewing medical plans as necessary, taking into account the impact on outcomes such as the quality of medical care in the region, and whether the group of policies linked to each item in the desired direction is effective as a whole. It is important to logically consider specific methods to solve the problems identified, include policies that are as effective as possible, and show the relationship between each policy and the issues to be solved, and to consider using tools such as logic models when examining and evaluating policies.

Prefectural governments are required to clarify the medical functions required for each of the five diseases, six programs, and home medical care, and then build a medical cooperation system with the cooperation of local medical professionals, etc., and by clearly showing it, patients and residents can understand the current situation of the functional division of each local medical institution and receive high-quality medical care appropriate to the stage of the disease. In addition, it is important for each prefecture to systematically work on human resource development and encourage employees to take training courses implemented by the national government in order to ensure the effectiveness of the PDCA cycle by using tools such as logic models.

In formulating medical plans, it is of great significance for the future progress of medical care that not only medical and administrative personnel but also patients (including their families) and residents have a common understanding of the current state of medical care and engage in unified discussions and deliberations to resolve issues. For this reason, prefectures should reflect the opinions of patients and residents by having them participate in working groups, holding town meetings, conducting interviews and questionnaire surveys of patients and residents, and soliciting public comments on medical plans, and then publish the contents of the medical plans in an easy-to-understand manner and share them with the surrounding areas.

It is necessary to know.

Section 2. General points to note when preparing a medical plan 1.

Legal procedures for preparing a medical plan

When creating a medical plan, etc., the following procedures based on the law must be followed.

- (1) In formulating a medical plan, the prefectures concerned may, when they deem it necessary in light of the actual conditions of medical supply and demand in areas around prefectural borders.

Coordinate and liaise with the relevant authorities.

- (2) In order to create medical plans, prefecture-level units were established.

Opinions will be heard from medical associations, dental associations, pharmaceutical associations and other groups of knowledgeable people related to medical treatment or dispensing.

(3) When formulating or amending a medical plan, the opinions of municipalities (including partial administrative associations and wide-area associations prescribed in Article 284, paragraph 1 of the Local Autonomy Law (Law No. 67 of 1947) which handle emergency medical services; the same applies hereinafter) and the insurers council shall be heard in advance.

(4) When formulating or amending a medical plan, the prefecture,
Listen to the opinions of prefectural medical councils.

(5) When a medical plan is established or changed, the Minister of Health, Labour and Welfare shall submit it to the Minister of Health, Labour and Welfare without delay and publicize the contents. (6)

When necessary for the creation of a medical plan and the implementation of measures, the Minister of Health, Labour and Welfare shall
We will request villages, government offices, medical insurers, and operators or managers of medical facilities to provide necessary information, such as information on medical functions.

2 Recorded matters

The following items must be included in the medical plan: (1) Goals to be achieved by prefectures for the five diseases, six programs, and home medical care

Matters concerning

(2) Matters related to the medical cooperation system for five diseases, six programs, and home medical care (3) Matters related to the promotion of information provision regarding medical functions in the medical cooperation system
item

(4) Matters relating to five diseases, six programs, and home medical care (5) Matters relating to regional medical planning (6) Matters relating to the promotion of the provision of information on the functions of hospital beds (7) Matters relating to outpatient medical care (8) Matters relating to securing physicians and medical professionals (excluding physicians) (9) Matters relating to ensuring the safety of medical care

(10) Matters concerning the designation of areas where hospital beds should be developed

(11) Matters concerning the standard number of hospital beds

(12) Provision of medical care taking into consideration the goals of establishing regional medical support hospitals and other medical functions
Matters concerning the goals of facility development

(13) Other matters necessary for ensuring the medical care system

3. Relationship with other plans

In formulating medical care plans, efforts must be made to ensure that they are in harmony with plans prescribed by other laws that stipulate matters concerning the assurance of medical care, and that they are coordinated with public health, pharmaceutical affairs, social welfare, and other policies closely related to medical care.

In addition, efforts must be made to ensure consistency with the comprehensive assurance policy and prefectural plans set out in the Comprehensive Medical and Nursing Care Assurance Act, as well as the basic policy, prefectural nursing care insurance business support plan, municipal nursing care insurance business plan, and prevention plan, and prefectural action plan set out in the Long-Term Care Insurance Act.

Examples of plans that include details regarding securing medical care and measures that are closely related to medical care include the following: In addition, if other plans that are closely related in terms of policy stipulate that similar details to those to be included in the medical care plan should be included, it is acceptable to substitute specific details by clearly indicating the corresponding parts of these plans in the medical care plan.

(1) The Act on Special Measures Concerning Support for Sustainable Development of Depopulated Areas (Act No. 19)

(2) Remote Island Development Plan based on the Remote Island Development Law (Law No. 72 of 1953)

* The Act to Partially Amend the Remote Islands Development Act (Act No. 92 of 2022), enacted on November 18, 2022, requires the national and local governments to take special measures to improve medical care in remote islands, and please note that telemedicine is clearly stated in the provisions for such measures.

(3) Mountain Village Development Plan based on the Mountain Village Development Law (Law No. 64 of 1965)

(4) Policies set out in Article 11 of the Basic Policy

• Basic policy set out in the Health Promotion Act (Act No. 103 of 2002) and the prefectural and municipal governments

Prefectural Health Promotion Plan

• Basic policy for rationalizing medical expenses as stipulated in the Law on Assurance of Medical Care for the Elderly and Prefectural Medical Cost Optimization Plans

• Guidelines for reducing doctors' working hours

• Cancer Control Promotion Fundamentals stipulated in the Cancer Control Basic Law (Law No. 98 of 2006)

This Plan and Prefectural Cancer Control Promotion Plans

γ The Basic Plan for Promoting Measures against Cardiovascular Disease and Prefectural Plans for Promoting Measures against Cardiovascular Disease, as stipulated in the Basic Act on Measures Related to Stroke, Heart Disease, and Other Cardiovascular Diseases to Promote Healthy Lifespans (Act No. 105 of 2018)

γ Act on Mental Health and Welfare of the Mentally Disabled (Act No. 123 of 1950)

Guidelines set out in

γ Basic guidelines for hepatitis countermeasures set forth in the Basic Act on Hepatitis Countermeasures (Act No. 97 of 2009)

Needle

γ Act on Medical Care for Patients with Intractable Diseases (Act No. 50 of 2014)

Basic policy set out in

γ Basic Law on Allergy Disease Countermeasures (Law No. 98 of 2014)

pointer

γ Basic policy, basic guidelines and prefectural welfare plans for disabled children as stipulated in the Child Welfare Act (Act No. 164 of

1947) γ Basic policy on developmental medical care as stipulated

in the Act on Comprehensive Promotion of Measures to Provide Necessary Developmental Medical Care to People in the Developmental

Process and Their Guardians, as well as Pregnant and Nursing Women (Act No. 104 of 2018)

γ Comprehensive suicide prevention measures as stipulated in the Basic Act on Suicide Prevention (Act No. 85 of 2006)

Guidelines and Prefectural Suicide Prevention Plans

γ The Basic Plan for Promoting Measures against Alcohol-Related Health Harm Measures and Prefectural Plans for Promoting

Measures against Alcohol-Related Health Harm Measures, as stipulated in the Basic Act on Measures against Alcohol-Related

Health Harm Measures (Act No. 109 of 2013)

γ As stipulated in the Law Concerning the Promotion of Dental and Oral Health (Law No. 95 of 2011)

Basic matters

γ The basic guidelines set out in the Law for Comprehensive Support of the Daily and Social Lives of Persons with Disabilities (Law

No. 123 of 2005) and Prefectural Disability Welfare Plans

painting

4. Establishment of a system for creating medical plans

In preparing various surveys and medical plans, we consult with relevant government agencies and medical institutions.

It is desirable to proceed with this project with sufficient cooperation among the relevant parties, such as by setting up a forum for discussion

with relevant organizations. In particular, the medical cooperation system for the five diseases, six programs, and home medical care

should involve doctors, dentists, pharmacists, nurses, other medical professionals, caregivers, etc.

It is important that the system be established through consultations between care service providers, patients, residents, and other local stakeholders as stipulated in the Nursing Care

Insurance Law.

The subsidy for the medical care delivery system promotion project (medical collaboration system promotion project) that is set up for this purpose can be used for establishing the system mentioned above.

5. Name of medical plan, etc. It is

desirable for the name of the medical plan in each prefecture to be "XX Prefecture Medical Plan," but so long as it is prepared in accordance with procedures pursuant to the law and contains matters pursuant to the law, it is acceptable for the plan to have a name such as "XX Prefecture Health and Medical Plan," and it is also acceptable for the plan to be a comprehensive plan that includes content in other related fields such as welfare.

6. Period of medical care plan

The medical care plan is surveyed and revised every six years, taking into account mid-term evaluations every three years. In addition, investigations, analysis and evaluations will be conducted every three years regarding matters related to home medical care, securing of doctors and outpatient medical care, and medical care plans will be revised if necessary. Article 3 Contents of Medical Care Plans The contents of medical care plans are generally expected to be as follows, but the specific contents,

including their structure, will be

decided by prefectures in accordance with the basic policy and in accordance with the actual conditions of their respective regions. However, as stipulated in Article 30-4, Paragraph 2 of the Law as items to be included in medical care plans,

Anything that is not listed must be included.

1. Basic Concept of Medical Plan

The basic principles of each prefecture in formulating medical plans
Write down.

(1) Purpose of creating medical plans

The legal basis for the medical plan and the purpose of its creation will be clearly stated.

(2) Basic Concept

The basic principles of each prefecture should be established while paying attention to consistency with the basic policy.
Write down.

(3) Positioning of medical plans

Create a comprehensive plan that includes content in other related fields such as health and welfare.

If the medical plan is being implemented, the relationship with the medical plan shall be clearly stated. (4) Period

of the medical plan The period covered by the plan shall be stated.

If a different period is covered, that period shall be noted.

The current situation of the region, which is the prerequisite for the medical plan, should be described. In addition to matters related to medical care, matters related to public health, pharmaceutical affairs, social welfare, and socio-economic conditions, etc. may be included.

For reference, the following are some possible indicators of the current situation in the region.

(1) Geography and Transportation

Regional characteristics, transportation conditions, geographical conditions, living area, etc.

(2) Demographic structure (including trends and future projections)

Population, three age groups, aging rate, number of households, etc.

(3) Demographics (including trends and future projections): Number of births, number of deaths, average life expectancy, etc.

(4) Residents' health status

(5) Status of residents receiving medical care Number of inpatients and outpatients, status of patients receiving medical

care within a secondary medical care area (meaning the area prescribed in Article 30-4, Paragraph 2, Item 14 of the Law; the same applies below) or within a prefecture (including the proportion of inflow and outflow patients), hospital bed occupancy rate, average number of residents Hospital days, etc.

(6) Status of medical facilities ÿ Hospitals

(number of facilities, number of beds by type) ÿ Clinics (number of clinics with and without beds, number of dental clinics, number of clinics with beds)

(Number of hospital beds)

3) Pharmacy

ÿ Other

3. Medical collaboration system for each of the five diseases, six programs, and home medical care

The medical collaboration system for each of the five diseases, six projects, and home medical care will be described in accordance with the policy outlined in Basic Policy No. 4 and in a manner that is easy for patients and residents to understand.

Specifically, for each of the five diseases, six services, and home medical care, (1) It should include (1) outcomes such as the health status of residents and the condition of patients, and the current situation regarding the medical care system, including patient trends and medical resources and collaboration; (2) the medical functions required to achieve the outcomes; (3) issues, numerical targets, and measures required to achieve the numerical targets; (4) in principle, the names of the medical institutions etc. responsible for each medical function; and (5) the method of evaluation and publication, etc.

In addition, when making the entries, please refer to (6) Public medical institutions, etc. and independent medical institutions, etc. Also, attention should be paid to the role of social medical corporations, and (7) hospital-hospital and hospital-clinic collaboration.

In addition, when necessary, the roles of related institutions, such as (8) the role of dental medical institutions (hospital dental clinics, dental clinics), (9) the role of pharmacies, and (10) the role of visiting nurse offices, should also be described. (1) Understanding the current situation

The current state of the local medical care system is described based on information on the health status of residents, the condition of patients (outcomes), trends in medical care, and information on medical resources and collaboration.

In addition, for the five diseases, six businesses and home medical care, the same system is in place for all prefectures.

Use indicators categorized into structure, process, and outcome

This will enable comparisons of the health status of residents and the medical care delivery system over time and between medical care areas, as well as the correlations between indicators related to the medical care delivery system.

(2) Necessary medical functions

For example, the medical functions required to establish a medical cooperation system will be clarified for each of the five diseases, six programs, and home medical care, including the age-adjusted mortality rate for each type of stroke and the medical functions required at each stage of the disease, from the acute and recovery stages to the maintenance and daily living stages. (3) Issues, numerical targets, and measures required to achieve the numerical targets

Regarding the five diseases, six services, and home medical care,

The current situation will be analyzed and issues in the regional medical care system will be identified.

Taking into account the goals set in each plan, etc.,

Based on this, each issue should be extracted, and then, based on the actual conditions of the region, a measurable and specific numerical target should be set, and the measures necessary to achieve the numerical target should be described. When considering measures, a cause analysis should be performed for the issue, and the perspective should be taken of how much impact the results (output) of the considered measures will have on the issue. In doing so, it is important to show the relationship between each measure and the issue to be resolved, and the use of tools such as logic models should be considered. (4) Specific

names of medical institutions, etc. As a

general rule, the names of medical institutions, etc. responsible for each medical function should be described. do.

In some areas, due to limitations on medical resources, may have multiple functions.

Regarding the names of medical institutions, etc., in cases where there are a significant number of medical institutions, etc. that perform each medical function within a medical cooperation system within a region, it is acceptable to separately include the URL of the website showing the name of the medical institution, etc. in the medical plan. (5)

Evaluation and Publication

Methods, etc. The evaluation and publication methods and review system for the five diseases, six projects, and home medical care shall be clarified. The annual trends in the numerical values of the target items and the progress of the policies shall be grasped and evaluated periodically (annually is preferable), and if the progress toward the targets is insufficient, the cause shall be analyzed and a report shall be made on the results (output) of the policies and projects as well as the health status of residents, the condition of patients, the quality of medical care in What impact did it have on the outcome?

It is necessary to review policies as necessary from this perspective. In doing so, it may be possible to reuse tools such as the logic model used in (3).

In addition, when conducting evaluations at prefectural medical councils, etc., In order to fulfill this role, the composition of the committee (doctors, dentists, pharmacists, (nurses, medical insurers, people receiving medical care (patients, etc.), people with academic and experience) and management (active use of working groups and committees representing patients)

(6) Roles of public medical institutions, etc., independent medical institutions, and social medical corporations

Based on the roles of public medical institutions, etc. (meaning medical institutions established by those listed in each item of Article 7-2, Paragraph 1 of the Law; the same applies below, except for Section 3-3 (6)) and medical institutions established by independent administrative agencies listed in Section 4-6 of the Enforcement Order of the Medical Care Act (excluding the National Institutes for Quantum and Radiological Science and Technology and the Independent Administrative Agency, the Japan Maritime Technical Education Agency; including national university corporations to which the provisions of Article 7-2, Paragraph 7 of the Law apply mutatis mutandis, as deemed to be independent administrative agencies specified in Article 4-6 of the Enforcement Order of the National University Corporation Act (Cabinet Order No. 478 of 2003)), the division of functions and business collaboration between medical care facilities is described below.

In particular, for public medical institutions such as public hospitals (meaning public medical institutions as defined in Article 31 of the Law), taking into consideration the medical needs of the region, etc., it is important to focus on functions that cannot be assumed by local private medical institutions. Based on this, the implementation status of operations related to the business of ensuring emergency medical care, etc. (meaning the business listed in Article 30-4, Paragraph 2, Item 5, A to F of the Law; the same applies below) that is required as part of the role of such public medical institutions will be clarified for each hospital.

In addition, based on the "Guidelines for strengthening public hospital management to ensure a sustainable regional medical care system" published by the Ministry of Internal Affairs and Communications (Attachment to the Notice of the Director-General of the Local Finance Bureau of the Ministry of Internal Affairs and Communications No. 72 dated March 29, 2022), we will take into consideration the division of roles and cooperation with regional medical institutions and strive to ensure consistency with the reorganization and networking of

public hospitals. In addition, social medical corporations will be actively involved in projects to ensure emergency medical care, etc. As the government is aiming to play a key role in all certified businesses, It is important to clarify that the institution is a social medical corporation.

Furthermore, in the event of the outbreak or spread of a new infectious disease, the prefectural governor, in accordance with the Infectious Diseases Act, will appoint public medical institutions, etc. (medical institutions established by those listed in each item of Article 7-2, Paragraph 1 of the Act, the National Hospital Organization, the Independent Administrative Institution, the Independent Medical ... Medical facilities established by the Japan Occupational Health and Safety Organization and other governmental organizations Act on Prevention of Infectious Diseases and Medical Care for Patients with Infectious Diseases

Based on the provisions of the Enforcement Regulations (Ministry of Health, Labour and Welfare Ordinance No. 99 of December 28, 1998) and the notification to regional medical support hospitals and designated function hospitals of their mandatory provision of emerging infectious disease medical care, their roles will be clarified in accordance with the actual conditions of each region.

(7) Hospital-hospital and hospital-clinic cooperation

In the future, in building a regional medical care system, it is planned to promote functional differentiation and collaboration of hospital beds in the regional medical care concept (meaning the regional medical care concept stipulated in Article 30-4, Paragraph 2, Item 7 of the Law; the same applies below). It is necessary to build an efficient and effective medical care system throughout the region, taking into account the role that each medical institution plays in the region. In medical care plans, it is necessary to work on building a seamless collaboration system from the acute phase to the recovery and chronic phases, and to build a system that includes disease prevention and nursing care prevention, so it is necessary to further promote hospital-hospital and hospital-clinic collaboration.

It will be necessary to do so.

In addition, in order to efficiently share patient medical information, etc., when coordinating with medical institutions and related institutions, such as hospital-hospital and hospital-clinic collaboration,

Consideration will also be given to the use of ICT.

Regarding local clinics, if they are able to provide emerging infectious disease medical care, they should conclude agreements based on the Infectious Diseases Act whenever possible, and it is important that medical institutions providing emerging infectious disease medical care cooperate with doctors and clinics who are familiar with patients on a daily basis, including clinics providing regular medical care other than emerging infectious disease medical care. Therefore, all medical institutions are obligated to respond to discussions regarding the conclusion of such agreements, and prefectures should take measures to promote the coordination of emerging infectious disease medical care and regular medical care in the region, utilizing surveys and discussions prior to the conclusion of such agreements. Confirm roles and encourage collaboration.

In addition, if local clinics are in charge of regular medical care other than emerging infectious disease care, they should endeavor to guide patients to appropriate medical facilities such as fever clinics in response to inquiries from patients. In doing so, they should advise the patient to inform the medical facility of their own underlying illness, the treatment they are receiving, and their medical history at the clinic, and to make use of their medicine notebook. In addition, the medical facility should use an online qualification confirmation system, etc. to verify the patient's My Number.

By obtaining the consent of patients who bring their card, medical treatment and drug information can be checked to provide more accurate information that is tailored to the patient.

It will be possible to do so.

(8) Role of dental medical institutions (hospital dental clinics, dental clinics)

In order to promote the establishment of a comprehensive community care system, it is important for dental medical institutions to take measures based on the actual conditions of the region, such as promoting collaboration with local medical institutions. In particular, since the relationship between the oral cavity and the whole body has been widely pointed out in recent years, such as the prevention of aspiration pneumonia by managing the oral cavity, it is necessary to clearly state the roles of dental medical care and dental medical professionals in establishing a system of medical collaboration, and to further promote medical-dental collaboration for hospitalized patients and patients receiving treatment at home. (9) Role of pharmacies

In order to provide safe and high-quality medical care in the community, it is important to build a system that allows drug therapy to be smoothly continued when patients move from hospitalization to outpatient and home medical care. For this reason, in addition to ensuring a supply system for pharmaceuticals, local pharmacies must provide a unified and continuous understanding of patients' medication information in cooperation with medical institutions, pharmaceutical management and guidance based on that information, cooperation with medical institutions when patients are admitted or discharged from hospital, dispensing at night and on holidays, and responding to telephone consultations.

It will be necessary for them to play an equal role in responding to the needs of the market.

(10) Role of visiting nurse offices

In order for people to live safely and healthily in familiar neighborhoods, it is necessary to provide uninterrupted medical services 24 hours a day and to be able to smoothly transition between medical institutions and homes. For this reason, the role of home-visit nursing offices, which provide services such as medical treatment and support for recuperation at home, is important. In an ageing society with a high death rate, it is particularly important to strengthen cooperation between home-visit nursing offices and related institutions, strengthen the functions of home-visit nursing offices by expanding the scale of their business, and improve operational efficiency by utilizing information and communication devices, in order to be able to provide end-of-life care and support for seriously ill patients, even at home.

It is necessary to establish a system for providing stable home nursing services through

In addition, the medical and nursing staff will also be responsible for providing support to pediatric patients who require medical care on a daily basis.

Strengthen cooperation with related organizations to provide welfare services.

It is necessary to do so.

4. Medical care that the prefectural governor deems especially necessary in light of the outbreak of disease, etc.

In light of the occurrence of diseases other than the five diseases and six businesses in the prefecture,

The prefectural governor must clearly state the medical care that he or she deems particularly necessary.

In formulating a regional medical plan and promoting differentiation and collaboration of hospital bed functions, the attached "Guidelines for formulating regional medical plans" (Annex 1 to the Notification of the Director-General of the Medical Affairs Bureau, Ministry of Health, Labour and Welfare, No. 0331-53, dated March 31, 2015) and "Regarding the promotion of regional medical plans" (Notification of the Director-General of the Regional Medical Planning Division, Medical Affairs Bureau, Ministry of Health, Labour and Welfare, No. 0207-1, dated February 7, 2018) should be taken into consideration.

6. Ensuring the medical care delivery system for outpatient medical care

When formulating outpatient medical care plans and promoting the differentiation and collaboration of functions among hospitals and clinics related to outpatient medical care in the region, the Guidelines for Ensuring the Medical Care Delivery System for Outpatient Medical Care (Announcement by the Director of the Regional Medical Planning Division and the Director of the Medical Affairs Division of the Medical Affairs Bureau, Ministry of Health, Labour and Welfare dated March 29, 2019, Medical Affairs Bureau Notification No. 0329-3 and Medical Affairs Bureau Notification No. 0329-6) must be taken into consideration.

7. Securing doctors and medical personnel (excluding doctors)

Regarding matters related to securing doctors and medical personnel, taking into consideration that the effort to build a medical cooperation system itself will be a measure to eliminate uneven distribution, that the degree to which doctors and medical personnel should be secured in a region will be affected by the policy on reorganization and integration of medical institutions in the regional medical plan, and that prefectures are required to take the lead in reconstructing the system for dispatching doctors to regional medical institutions, a council will be held to determine necessary measures for matters related to securing medical personnel pursuant to the provisions of Article 30-23, Paragraph 1 of the Law (hereinafter referred to as the "Regional Medical Measures Council"), and the specific measures decided by said council will be described here.

(1) Securing doctors

Regarding securing doctors, see the Doctor Securing Plan Formulation Guidelines (2019)

March 29, Medical Affairs and Local Government Notification No. 0329-3 and Medical Affairs and Medical Affairs Notification No. 0329-6, Ministry of Health, Labor and Welfare

Based on the notification by the Director of the Regional Medical Planning Division and the Director of the Medical Affairs Division of the Political Affairs Bureau (attached), the formulation of the plan and

and to implement the measures.

(2) Securing medical personnel other than doctors

For medical professionals other than physicians, such as those in the following professions, the current situation and goals for securing medical professionals should be described as specifically as possible, including matters related to improving their qualifications as necessary.

[Current situation and goals of medical professionals] ÿ

Dentists ÿ

Pharmacists ÿ Nursing staff (public health nurses, midwives, nurses (nurses who have completed specific practice training) (including nurses) and licensed practical nurses)

ÿ Other health and medical workers

Radiological technologists, clinical laboratory technologists, physiotherapists, Occupational therapists, orthoptists, speech-language-hearing therapists, clinical engineers, prosthetists, paramedics, dental hygienists, dental technicians, registered dietitians, etc.

ÿ Care service workers

In particular, when writing about dentists, pharmacists, and nursing staff,

The following points should be taken into

consideration: A. With regard to dentists, it is desirable to clarify the role of dentistry in hospitals in order to further promote medical-dental cooperation, taking into account the widely-recognized relationship between the oral cavity and the entire body. Specifically, after grasping the deployment situation of dental medical professionals in the region, it is possible to recognize the role of dentistry in medical-dental cooperation, and to promote initiatives based on the actual conditions of the region, such as utilizing local dental medical professionals in hospitals according to the size and function of the hospital, and promoting cooperation between hospitals and dental clinics. In addition, the Regional Medical Care and Nursing Comprehensive Security Fund should be actively utilized to further promote these initiatives.

thing.

Regarding pharmacists, in order to contribute to ensuring the effectiveness and safety of drug therapy in community medical care and to improving and promoting public health, in addition to dispensing and other duties, hospital pharmacists are being asked to further enhance their duties and roles, mainly focusing on ward pharmaceutical duties and team medical care, while pharmacy pharmacists are being asked to further enhance their duties and roles, mainly focusing on home medical care and advanced pharmaceutical management.

In light of the uneven distribution of pharmacists across different business types and regions, and the particular shortage of hospital pharmacists being an urgent issue, in order to secure the necessary number of pharmacists, the employment status of pharmacists in hospitals and pharmacies should be grasped, and measures to secure pharmacists that correspond to the actual conditions of the region, including active use of the Regional Medical Care and Nursing Comprehensive Security Fund (study loans, dispatching pharmacists to medical institutions, etc.), should be described as specifically as possible. When considering and implementing measures to secure pharmacists, the pharmaceutical affairs division and medical affairs division of the prefecture, as well as related organizations such as the prefecture's pharmaceutical association, should work together. In particular, when considering and implementing measures to secure hospital pharmacists, the prefecture's hospital pharmaceutical association should also work together.

In addition, in order to improve their qualifications, based on the "Pharmacy Vision for Patients" (PSEHB Notification No. 1023-3, issued by the Director of the General Affairs Division, Pharmaceutical and Food Safety Bureau, Ministry of Health, Labour and Welfare, on October 23, 2015), pharmacy staff will be required to understand the status of training implementation and make adjustments among relevant parties so that training that contributes to improving communication skills with patients and residents and training conducted jointly with multiple professions that will lead to stronger collaboration with medical institutions, etc., will be conducted based on the acquisition of specialized information on the latest medical care and pharmaceuticals, etc.

Regarding nursing personnel, in order to secure them, we will work with prefectural nurse centers and other relevant parties to grasp the issues in each prefecture and secondary medical area, and promote efforts based on the three pillars of new training, support for returning to work, and promotion of retention, such as training by nursing training institutes, support for returning to work by prefectural nurse centers using the "human resource utilization system for nurses using the My Number system" and notifications of nurses leaving their jobs, and prevention of resignations by improving the working environment at medical institutions.

In addition, in order to respond to the increasing demand for visiting nurses in the region, Based on the actual situation in the region, the use of the Regional Medical Care and Nursing Comprehensive Security Fund and the Engage in visiting nursing, including enhancing initiatives at the Prefectural Nurse Center. Describe measures to secure nursing staff.

In addition, with regard to nurses, in order to systematically train nurses in the region who can support home medical care and who can respond quickly and appropriately in the event of an outbreak or spread of an infectious disease, the government will implement measures to ensure that nurses are given special training based on the actual conditions of the region.

In order to enable people to take designated practice training (meaning specific practice training as stipulated in Article 37-2, Paragraph 2, Item 4 of the Public Health Nurse, Midwife and Nurse Act (Act No. 203 of 1948)) locally, the plan for establishing a training system, such as securing designated training institutions and cooperating facilities for practical training, must be specifically described. In addition, the target number of employed nurses who have completed the specific practice training and who have a high level of expertise must be stated. When setting these target numbers, it is recommended to consider setting them by secondary medical area and by field/area as much as possible. thing.

8. Ensuring medical safety The

current status and goals of medical facilities and medical safety support centers shall be described in accordance with (1) and (2).

- The above points should be

included in the medical plan so that it can be easily understood by local patients and residents.

thing.

- In doing so, prefectures, in cooperation with cities and special wards that have public health centers, will grasp the status of efforts to ensure medical safety at medical facilities, including efforts to respond to medical accidents (meaning medical accidents as defined in Article 6-10, Paragraph 1 of the Law), and will plan the current status and goals of the prefectures' efforts to provide information, conduct training, raise awareness, etc. regarding medical safety.

This must be clearly stated in the picture.

- In addition, in order to build a system for responding to complaints and inquiries about medical care from patients or their families in local communities and providing necessary advice to medical care facilities as necessary, the prefectural government will implement measures to support medical safety in the prefectures.

The plan must also clearly state the status of the center's establishment and its goals.

- Check the placement and operation status of advanced medical equipment in hospitals,

Conduct an evaluation, including maintenance inspections.

- Maintenance and inspection of medical equipment such as CT and MRI for clinics that have such equipment

Require regular reports on the status of medical safety efforts, including

(1) Current status of measures to ensure the safety of medical care at medical care facilities

Status and goals

ý The ratio of medical facilities to the total number of hospitals, general medical clinics, dental clinics, and midwifery clinics

Percentage of medical facilities that have a medical safety manager

ý The ratio of hospitals that have full-time or dedicated medical safety managers to the total number of hospitals. ý The ratio of hospitals that have full-time or dedicated medical safety managers to the total number of hospitals, general medical clinics, dental clinics, and midwifery clinics.

Percentage of medical facilities that have established consultation desks for medical safety

ý The total number of hospitals, general medical clinics, dental clinics, and midwifery clinics.

Percentage of medical facilities whose managers have attended training on the medical accident investigation system (limited to those held by the Medical Accident Investigation and Support Center or the Liaison Council of Support Groups (including those commissioned to do so))

ý The percentage of hospitals that have been evaluated by other hospitals for their medical safety measures or that have undergone third-party evaluation (limited to hospital function evaluations conducted by the Japan Council for Quality Health Care, evaluations based on JCI accreditation conducted by the Joint Commission International, and evaluations based on ISO 9001 accreditation based on ISO standards) out of the total number of hospitals

(2) Current status and goals of the Medical Safety Support Center

When filling out the form, please refer to the "Implementation of Medical Safety Support Centers" (Notice No. 0330036 from the Director-General of the Medical Affairs Bureau of the Ministry of Health, Labour and Welfare dated March 30, 2007) and fill out the following items: 1) The ratio of the number of secondary medical care areas that have established medical safety support centers to the total number of secondary medical care areas

ý Number of consultation staff (full-time equivalent) ý The ratio of the total number of consultation staff to the total number of medical safety support centers

Percentage of consultation staff who have attended the training provided

ý Information on the activities of medical safety support centers in prefectures, secondary medical districts, cities with public health centers, and special wards through websites, public relations, etc.

Status of information provision

ý Medical Safety Support Centers that provide training on medical safety to medical personnel in prefectures, secondary medical districts, cities with public health centers, and special wards.

Percentage of centers

ý Patients in prefectures, secondary medical care areas, cities with public health centers, and special wards

- Status of awareness-raising activities to promote medical safety among residents

ý Status of establishment and holding of medical safety promotion councils in prefectures, secondary medical care areas, cities with public health centers, and special wards

9. Standard number of

hospital beds (1) Medical beds and general hospital beds

The standard number of hospital beds for medical care and general hospital beds shall be calculated for each secondary medical care area based on the formula set forth in Article 30-30, Paragraph 1 of the Ordinance.

and.

On average nationwide, more than 90% of patients live in the prefecture where they live.

Considering the current situation of inpatient treatment, adjustments can be made between prefectures only when it is particularly necessary. In such cases, the calculation of the standard number of hospital beds will use the value based on the location of the medical institution.

(2) Psychiatric beds, tuberculosis beds and infectious disease beds

The standard numbers of psychiatric beds, tuberculosis beds and infectious disease beds shall be calculated for each prefectural area based on the formulas set out in Article 30-30, Items 2 to 4 of the Ordinance.

In addition, even if two or more areas as stipulated in Article 30-4, Paragraph 2, Item 15 of the Law (hereinafter referred to as "tertiary medical care areas") are established in one prefecture, the standard number of hospital beds shall be determined for the entire prefecture.

(3) Calculation of the number of in- and out-patients in each area

When calculating the number of in- and out-patients in each area, if hospitals are specifically asked to submit reports, the purpose of creating medical plans, etc. will be fully explained to the hospitals being surveyed, and consideration will be given to ensuring smooth administrative processing.

thing.

(4) Exceptions to the calculation of the standard number

of hospital beds When there is a situation specified in Article 5-2 of the Enforcement Order of the Medical Care Act, such as a sudden increase in the population of each district, the prefectural governor, in cooperation with the Minister of Health, Labour and Welfare, may

The standard number of hospital beds may be determined based on the number discussed and agreed upon.

(5) Prefectural Governor's Recommendation

After the standard number of hospital beds is calculated based on (1) to (3), each prefecture

Opening a hospital, increasing the number of hospital beds or changing the type of hospital beds in

Application for permission to establish or increase the number of beds in a clinic (hereinafter

In the case where a license application, etc. is filed, and the number of existing hospital beds in the area exceeds the standard number of hospital beds, or the number of hospital beds will exceed the standard number of hospital beds as a result of the license application, etc., the hospital may be subject to a recommendation from the prefectural governor pursuant to the provisions of Article 30-11 of the Law (hospitals, etc. opened by public medical institutions, etc., may be subject to a disapproval disposition pursuant to the provisions of Article 7-2, Paragraph 1 of the Law).

thing.

10. Goals for the development of medical facilities

Community medical support hospitals are hospitals approved by prefectural governors as hospitals that, as part of the systematization of medical facility functions, play a role in supporting regions with a shortage of doctors, provide medical care to referred patients, share medical equipment, and have the capacity to support family doctors and dentists through training to improve the quality of local medical workers, and have structures and facilities appropriate for a hospital that ensures community medical care.

The development goals of the hospital will be considered taking into account the following functions and local conditions.

• Hospital-clinic cooperation system, such as referrals from family doctors, etc. •

Shared use status • Emergency

medical system • Training for

improving the qualifications of medical professionals, such as lifelong education

system

In addition, based on the provisions of Article 9-19, Paragraph 1, Item 2 of the Ordinance, matters that the prefectural governor determines as being particularly necessary for the hospital to perform in order to ensure medical care in the region shall be reviewed and publicized as necessary when formulating and reviewing the medical care plan. In addition, in light of the fact that the amendment of the Infectious Diseases Act by the Act partially amending the Infectious Diseases Prevention and Medical Care for Patients Act, etc. requires regional medical support hospitals to provide medical care in the event of an outbreak or spread of infectious diseases, when setting goals for the development of regional medical support hospitals, the matters regarding infectious diseases in the medical care plan shall be reviewed and publicized as necessary.

It is important to coordinate with other issues related to responses to outbreaks and epidemics.

Regional medical support hospitals have the function of treating referred patients and may also be designated as priority medical institutions for referrals.

Based on the results of these studies, targets for the development of regional medical support hospitals (for example, to develop hospitals in each secondary medical area) should be set as necessary.

In addition, in secondary medical care areas that do not have regional medical support hospitals,

It is important to improve the division of functions and business collaboration between the relevant agencies.

11. Other matters necessary for ensuring a system for providing medical care

Regarding the five diseases, six programs, and diseases other than home medical care, after grasping the current situation regarding patient trends and medical resources, etc., please enter the medical care, etc. that you deem especially necessary in light of the disease situation in the prefecture, taking into consideration the following points.

In addition, we will endeavor to include the names of medical institutions that provide medical care for each disease, etc.

To do.

(1) Health measures for people with disabilities

• Efforts to ensure medical care for people with disabilities (including people with higher brain dysfunction and developmental disabilities) (securing specialized medical institutions in prefectures, establishing a system of cooperation with related institutions, etc.)

• Contact points for consultations,

etc. (2) Measures against tuberculosis and infectious diseases

• Roles of each medical facility in tuberculosis and infectious disease control • Measures against influenza, AIDS, hepatitis, etc. (3) Measures against transplantation medicine

(organ transplantation, hematopoietic stem cell transplantation) • Measures by

prefectures • Contact points for

consultation, etc.

(4) Measures for intractable diseases

• Prefectural initiatives for rheumatism, etc. • Contact points for consultation, etc.

(5) Measures against allergic diseases

• Prefectural Initiatives

• Contact information for consultation etc.

(6) Measures for chronic obstructive pulmonary disease (COPD)

• Prefectural Initiatives

ý Contact information for
consultation, etc. (7) Measures for chronic kidney
disease (CKD) ý Prefectural
initiatives ý Contact information for
consultation, etc. (8) Measures for diseases that will increase with the aging population in the future
1) Measures against locomotive syndrome, frailty, etc.
Measures against locomotive syndrome, frailty, and femoral neck fractures
importance
Comprehensive prevention, medical care, and nursing care measures for locomotor syndrome, frailty, and femoral
neck fractures
ý Prefectural initiatives for aspiration pneumonia, etc. (9) Dental
health care measures ý Prefectural initiatives
ý Contact points for consultation,
etc. (10) Measures for securing
blood and appropriate use ý Prefectural initiatives ý Contact
points for consultation, etc. (11)
Measures for appropriate use of
pharmaceuticals, etc. ý Prefectural initiatives ý Contact
points for consultation, etc. ý Status
of clinical trials and pharmaceutical
supply system
(12) Information technology in medical care ý
Popularization of information systems in medical facilities (electronic receipts, medical records, etc.)
Status and Initiatives
ý Efforts to share information between medical institutions and related organizations using information and communication
technology (ICT) (including information security measures).
(13) Comprehensive efforts in health, medical care, and nursing care (welfare) In order
to ensure a regional medical care system, it is necessary to ensure that a variety of services that meet the needs of the
region, from disease prevention to treatment and nursing care, are seamlessly and consistently provided in the region.
The basis for providing medical care should be to establish patient-centered care.
For this reason, we are involved in disease prevention, nursing care, public health, pharmaceutical affairs, social welfare and other medical care.

For closely related policies, describe the methods of collaboration and the system for providing information to local residents.

As for measures closely related to medical care, the plan listed in Section 2.3 is as follows:

12. Evaluation and review of policies

Regarding the implementation status of policies, prefectures are required to verify the achievement status of policies based on the set numerical targets, etc., and to reflect the results in the next review of medical care plans. In doing so, they may also consider conducting analyses with the cooperation of academic experts.

Defeat.

The evaluation and review of measures based on Article 30-6 of the Law is as follows:

(1) Policy goals, etc. Numerical targets for the medical cooperation

system for the five diseases, six

programs, and home medical care (2) Promotion system and roles

The promotion system for achieving the policy goals and the responsibilities and roles of the relevant parties

(3) The time required to achieve the goal; and (4)

The measures to achieve the goal.

(5) Evaluation and review

(6) Methods for publicizing and disseminating the progress and evaluation

results Section 4. Procedures for formulating

medical plans For reference, the procedures that are considered to be common nationwide from a technical standpoint when prefectures formulate medical plans are shown below. 1.

Outline of the procedures for formulating medical plans

When preparing a medical plan, the following steps can be considered:

(1) Establishment of a system for preparing a medical plan (draft) (see Section 2, Paragraph 4) (2) Review of the objectives and basic principles of the medical plan and the basics of the medical plan

Consideration of the outline

(3) Verification of the effectiveness of measures implemented based on the current medical plan

(4) Consideration of medical areas and standard number of hospital beds

(5) Establishment of medical cooperation systems for each of the five diseases, six programs, and home medical care

Consideration of issues, numerical targets, and measures involved in the construction

(6) Establishment of a medical cooperation system for each of the five diseases, six programs, and home medical care

Construction Considerations

(7) Consideration of measures for formulating a regional medical plan and promoting differentiation and cooperation of hospital bed functions. (8)

Consideration of securing a medical care system for outpatient care. (9) Consideration of securing doctors and medical personnel (excluding doctors).

Consider

(10) Consideration of ensuring medical safety and other necessary matters. (11) Preparation of a medical plan (draft) based on the above considerations. (12) Hearing opinions on the medical plan (draft) from organizations of academic experts in medical treatment or dispensing (medical associations, dental associations, and pharmaceutical associations) (revise the draft if necessary).

(13) Deciding on the medical plan (draft) (14)

Hearing the opinions of municipalities and the insurer council regarding the medical plan (draft)

(Revise the medical plan (draft) as necessary)

(15) Consult with and report to the Prefectural Medical Council on the medical plan (draft) (16) Decide on the medical plan (17) Submit the medical

plan to the Minister of Health, Labour and Welfare and make it public *Efforts will

be made to provide necessary information from the contents of the medical plan to residents in an easy-to-understand format, such as by creating an overview version for residents and adding explanations of terms. In doing so, efforts will be made to obtain the understanding and cooperation of residents by indicating issues and prospects for the local medical care system from the perspective of using limited medical resources effectively. 2. Method of establishing medical areas

(1) In establishing secondary medical care areas, the following should be determined in consideration of natural conditions such as geographical conditions, and social conditions such as the degree of satisfaction of daily life needs and transportation conditions.

As a part of the body, medical care related to hospitalization in hospitals (provided in tertiary medical care areas) (excluding cases where it is considered appropriate to do so)

Areas where it is deemed appropriate will be certified as units.

However, the following points may be helpful in this regard.

• Population structure, patient medical care situation (proportion of inflow and outflow patients)

Regarding basic matters related to health demand and the supply of health care, such as the distribution of medical facilities and the distribution of medical facilities, consideration should be given to displaying them on a map by secondary medical area or by city, town, or village. In addition, in the case of secondary medical areas with a population of 1 million or more, in light of the fact that there are many cases where issues arise in operating them as planned areas, consideration should be given to revising the definition of the areas as necessary, and operational ingenuity should be given to holding regional medical plan coordination meetings by dividing the planned areas into smaller areas or by discussion items such as regional medical issues. In addition, in order to grasp the status of patients' medical treatment, it is desirable to conduct various surveys using statistically significant methods in addition to utilizing patient surveys.

Regarding secondary medical care areas with a population of less than 200,000, if they are not considered to function as an integrated area providing medical care related to hospitalization (especially if the inflow rate of patients is less than 20% and the outflow rate of patients is 20% or more), the area should be reviewed. When reviewing the area, it is necessary to take into account the area size of the secondary medical care area and the access time to the main hospital.

If you do not change the settings, you can change the reason (geographical conditions, Please clearly state the area of the area, geographical access, etc.

Use as reference materials materials related to existing areas, i.e., municipal areas, jurisdictions of prefectural administrative agencies such as health centers and welfare offices, school districts (especially areas related to high schools), etc.

(3) It is appropriate to align secondary medical districts with the planned districts. (2) The districts in which medical cooperation systems for the five diseases, six projects, and home medical care are to be established should be flexibly set according to the actual conditions of patients' movements and local medical resources, without being limited to the traditional secondary medical districts.

(3) Tertiary medical care areas are generally established on a prefectural level.

However, in cases where the area is particularly large or there are other special circumstances, Within prefectures, it is acceptable for multiple tertiary medical care areas to be established.

In addition, medical care that is generally considered appropriate to be provided in tertiary medical care areas includes, for example, the following, which require special diagnosis or treatment:

To be able to.

(i) Medical care requiring advanced technology, such as organ transplants; (ii)

Medical care requiring the use of specialized medical equipment, such as hyperbaric oxygen therapy; (iii)

Medical care for diseases with a low incidence rate, such as congenital biliary atresia; (iv) Emergency

medical care requiring particularly specialized care, such as extensive burns, amputations of fingers and limbs, and acute

poisoning. (4) In light of the actual supply and demand of medical care in areas around prefectural borders, when it is reasonable

and appropriate to establish medical areas that include the areas of neighboring prefectures, it is acceptable to establish

medical areas that span multiple prefectures, provided that this is clearly stated in the plans of each prefecture.

In such cases, sufficient discussion and coordination should be carried out between the relevant prefectures, and the

Ministry of Health, Labour and Welfare should also be notified if necessary. Even if

a medical district is not established that includes the areas of neighboring prefectures, if cooperation with neighboring

prefectures is to be established in establishing a medical care delivery system, discussions should be held with the prefecture

with which the cooperation will take place, and specific details should be included in the medical care plan.

(5) Regarding the establishment of medical care areas, taking into consideration that secondary medical care areas are the units

of measures in the outpatient medical care plan and the doctor recruitment plan, discussions should be held in advance in

the formulation of the medical care plan, and if the settings are to be changed, the status of the review should be clearly stated.

To report the situation to the government in advance.

3. Calculation method for standard number of hospital beds

(1) Method of calculating the standard number of hospital

beds The calculation of the standard number of hospital beds shall be based on the following method (Article 30 of the Ordinance).

30

γ The standard number of medical beds and general beds shall be the total number calculated using the formula in (a) and the

number calculated using the formula in (b) plus or minus the number calculated using the formula in (c).

A. Medical beds

{(Sum of (the district's population by sex and age group) x (national average rate of admission to long-term care

beds by sex and age group) - (number that can be accommodated by nursing care facilities and home medical care, etc.)

+ (number determined by the governor within the range of 0 to the number of inflow hospitalized patients from other

districts to the district) - (number of inflow hospitalized patients from the district to other districts)

(Number determined by the governor within the range of the number of outflow inpatients) $\times$ (1/hospital bed occupancy rate)

B. General hospital beds

$\{(\text{Sum of (Population by sex and age group in the district)} \times (\text{General hospital bed discharge rate by sex and age group in the district}) \times \text{Average length of hospital stay} + (\text{Number determined by the governor within the range of 0 to the number of inflow inpatients from other districts to the district}) - (\text{Number determined by the governor within the range of 0 to the number of outflow inpatients from the district to other districts})\} \times (1/\text{Hospital bed occupancy rate})$ C. Inter-prefectural

adjustment of standard number of hospital beds

In addition, if the number of inpatients flowing out of the prefecture is greater than the number of inpatients flowing into the prefecture, the prefecture may, after holding a coordination meeting with the receiving prefecture, add or subtract the number of agreed-upon numbers for the inflow and outflow of patients between prefectures to the standard number of hospital beds for each secondary medical care area.

can.

However, the number of hospital beds calculated for each secondary medical area based on A to C shall be The total number in prefectures is:

$\{(\text{Sum of (the district's population by sex and age group)} \times (\text{national average rate of admission to long-term care beds by sex and age group}) - (\text{number of beds that can be accommodated in nursing care facilities and at-home medical care, etc.})) \times (1/\text{hospital bed occupancy rate}) + \{(\text{Sum of (the district's population by sex and age group)} \times (\text{general hospital bed discharge rate by sex and age group}) \times \text{average length of hospital stay}) \times (1/\text{hospital bed occupancy rate})\}$ and the total number of hospital beds in each prefecture calculated for each secondary medical care area based on (c) above.

It cannot be exceeded.

(Note 1) "Population" refers to the night-time population at the time of creating the medical care plan. The

figures are based on the population results of the national census and the figures of local governments.

Based on the most recent official population statistics

Let us assume that.

(Note 2) "Age group" refers to age groups in five-year increments.

(Note 3) "National average rate of hospitalization in long-term care beds by gender and age group"

The number of patients admitted to hospitals by sex and age group for medical care beds as determined by the Minister of Health, Labour and Welfare.

The rate of patients receiving medical care was set as the upper limit, and chronic function was determined based on the regional medical care plan.

The number of patients that can be accommodated by nursing care facilities and home medical care, etc. is the number calculated by proportionally estimating the number equivalent to the total number of the following as of the end of FY2019 from the total number of the following numbers among the "future medical care needs at home, etc. in the planning area" set out in the regional medical planning: (i) Chronic inpatients who reside in the planning area

This number is equivalent to 70% of the number of patients in medical category I.

(ii) The number of chronic care inpatients who reside in the relevant planning area and who are estimated to have demand for home medical care, etc. by eliminating regional disparities in the rate of hospitalization (excluding the number listed in (i)).

(Note 5) "General bed discharge rate by sex and age group in the area" refers to the discharge rate by sex and age group in the regional block. (Note 6) "Bed occupancy rate" refers to the bed occupancy rate of long-term care beds or general beds set by the Minister of Health, Labour and Welfare, if it is lower than the most recent bed occupancy rate of long-term care beds or general beds in each prefecture, it shall be a value determined by the prefectural governor taking into consideration the situation of the prefecture, within the range of the bed occupancy rate of long-term care beds or general beds set by the Minister of Health, Labour and Welfare or higher and the most recent bed occupancy rate of long-term care beds or general beds in the area.

In addition, "the most recent occupancy rate of medical beds or general hospital beds in the area" In principle, the most recent data available should be used. However, due to the impact of COVID-19, it may not be appropriate to use the most recent data as is. In such cases, it is acceptable to use the "hospital bed occupancy rate in 2019" or the "average hospital bed occupancy rate from 2016 to 2019," which are not affected by COVID-19.

(Note 7) "Hospitalization rate for long-term care beds," "Discharge rate for general hospital beds," and "Bed availability" The figures used (as a reference) for "utilization rate" and "average length of hospital stay" are based on the figures used to calculate the standard number of medical beds and general beds under Article 30-4, Paragraph 2, Item 17 of the Law (Ministry of Health and Welfare Notification of 1986) This is stipulated in National Convention No. 165.

(Note 8) The number of inpatients admitted to each district is determined by the prefectural governor.

There are circumstances that the ministry deems necessary to ensure medical care in the area.

In such cases, the number of people in each area will be exceeded and a number that takes into account the circumstances of the area will be added.

It is possible to do so.

(Note 9) The number of inflow and outflow inpatients in each area is based on patient surveys,

This will be determined through surveys of National Health Insurance receipts, etc.

(Note) "Regional blocks" refers to the following nine blocks:

Block Name	Prefecture name
Hokkaido	Hokkaido
northeast	Aomori, Iwate, Miyagi, Akita, Yamagata, Fukushima
Kanto	Ibaraki, Tochigi, Gunma, Saitama, Chiba, Tokyo, Kanagawa, Niigata, Yamanashi, Nagano
Hokuriku	Toyama, Ishikawa, Fukui
Tokai	Gifu, Shizuoka, Aichi, Mie
Kinki	Shiga, Kyoto, Osaka, Hyogo, Nara, Wakayama
China	Tottori, Shimane, Okayama, Hiroshima, Yamaguchi
Shikoku	Tokushima, Kagawa, Ehime, Kochi
Kyushu	Fukuoka, Saga, Nagasaki, Kumamoto, Oita, Miyazaki, Kagoshima, Okinawa

y The standard number of psychiatric beds for each prefecture is calculated according to the following formula:

The number calculated above shall be used as the standard.

$$\left[\frac{\text{Number of acute inpatients by age in the relevant prefectures in 2026}}{\text{Total}} + \frac{\text{Recovery inpatients by age in the prefecture in 2026}}{\text{Total}} + \frac{\text{total number of chronic hospitalizations by age in the prefecture in 2026}}{\text{Total number of patients}} \right] \times \{1 - (\text{Percentage of the policy effect on chronic inpatients}$$

$$+ \frac{\text{number of people admitted to chronic dementia by age in the prefecture in 2026}}{\text{Total number of hospitalized patients}} \times \{1 - (\text{Policy effect on chronic dementia patients}$$

$$+ \frac{\text{number of people admitted to chronic dementia by age in the prefecture in 2026}}{\text{Total number of hospitalized patients}} \times \{1 - (\text{Policy effect on chronic dementia patients}$$

$$+ \frac{\text{number of people admitted to chronic dementia by age in the prefecture in 2026}}{\text{Total number of hospitalized patients}} \times \{1 - (\text{Policy effect on chronic dementia patients}$$

$$+ \frac{\text{number of people admitted to chronic dementia by age in the prefecture in 2026}}{\text{Total number of hospitalized patients}} \times \{1 - (\text{Policy effect on chronic dementia patients}$$

$$+ \frac{\text{number of people admitted to chronic dementia by age in the prefecture in 2026}}{\text{Total number of hospitalized patients}} \times \{1 - (\text{Policy effect on chronic dementia patients}$$

$$+ \frac{\text{number of people admitted to chronic dementia by age in the prefecture in 2026}}{\text{Total number of hospitalized patients}} \times \{1 - (\text{Policy effect on chronic dementia patients}$$

$$+ \frac{\text{number of people admitted to chronic dementia by age in the prefecture in 2026}}{\text{Total number of hospitalized patients}} \times \{1 - (\text{Policy effect on chronic dementia patients}$$

$$+ \frac{\text{number of people admitted to chronic dementia by age in the prefecture in 2026}}{\text{Total number of hospitalized patients}} \times \{1 - (\text{Policy effect on chronic dementia patients}$$

(Note 1) "Acute inpatients", "Recovery inpatients", "Chronic inpatients" and "Chronic dementia inpatients" are defined as follows:

do.

Acute inpatients: Inpatients who have been admitted to a psychiatric bed for less than three months.

Hospital Patient

Convalescent inpatient: Patients who have been hospitalized in a psychiatric bed for more than three months but less than one year

Inpatients who are

Chronic inpatients: Patients who have been hospitalized in a psychiatric bed for one year or more

Inpatients without dementia

Chronic dementia inpatients: Inpatients with dementia who have been hospitalized in a psychiatric bed for one year or more

(Note 2) In Notes 2 to 7, the "rate of change in age X (or older)" refers to the ratio of the number of inpatients nationwide in acute phase/recovery phase/chronic phase/chronic phase/chronic phase dementia aged X + 3 (or older) in 2017 to the number of inpatients nationwide in acute phase/recovery phase/chronic phase/chronic phase dementia aged X (or older) in 2014

(Note 3) "The number of acute

inpatients by age in the prefecture in 2026" refers to the "estimated number of acute inpatients by age who reside in the prefecture in 2026," calculated according to the following 1) to 4).

* The approximate number of acute inpatients of a given age who reside in the given prefecture in 2020 was calculated by dividing the number of acute inpatients of a given age group who reside in the given prefecture by 5.

Let it be a number.

1) The estimated number of patients of each age group from 0 to 24 years old is the number of acute care inpatients of that age who reside in the prefecture in 2020.

This is an approximate number of hospital patients.

2) The estimated number of patients between 25 and 27 years of age is calculated by multiplying the approximate number of acute inpatients who reside in the prefecture in 2020 and whose age is three years less than the age of the patient by the rate of change for that age.

The number obtained by multiplying is the number.

* The estimated number of patients in each age group between 25 and 27 years old in 2026 is calculated by multiplying the estimated number of patients in each age group between 22 and 24 years old in 2023 by the rate of change for that age group. In addition, the estimated number of patients in each age group between 22 and 24 years old in 2023 is calculated based on the idea in 1) above, and is the approximate number of acute inpatients of that age who reside in the prefecture in 2020. Based on these, the estimated number of patients in each age group between 25 and 27 years old in 2026 is calculated by multiplying the approximate number of acute inpatients of that age group between 22 and 24 years old who reside in the prefecture in 2020 by the rate of change for that age group.

3) The estimated number of patients in each age group from 28 to 89 years old is as follows:

The number obtained by multiplying b by c shall be the number obtained by multiplying that by ha.

The approximate number of acute inpatients in the prefecture who reside in the prefecture and whose age is the age minus six years (referred to as the "reference age" in (b) and (c)) in the year 2020

B. Percentage change in base age C.

Percentage change in age obtained by adding three years to the base age

* The estimated number of patients in each age group between 28 and 89 years old in 2026 is calculated by multiplying the estimated number of patients in each age group between 25 and 86 years old in 2023 by the rate of change for that age group. Similarly, the estimated number of patients in each age group between 25 and 86 years old in 2023 is calculated by multiplying the approximate number of acute inpatients in each age group between 22 and 83 years old in 2020 who reside in the prefecture by the rate of change for that age group.

We calculate it as follows.

4) The estimated number of patients aged 90 years or older is the sum of A and B multiplied by C.

The number obtained by this method shall be used.

B. The total number of the following items a through c

a The number obtained by multiplying the approximate number of acute inpatients aged 84 and residing in the prefecture in 2020 by the rate of change in that age group.

b The number obtained by multiplying the approximate number of acute inpatients aged 85 and residing in the prefecture in 2020 by the rate of change in that age. c The number obtained by multiplying the approximate number of acute inpatients aged 86 and residing in the prefecture in 2020 by the rate of change in that age. b The number obtained by multiplying the approximate number of acute inpatients aged 87 or older and residing in the prefecture in 2020 by the rate of change in the age group 87 or older. c The rate of change in the age group 87 or older.

* The estimated number of patients aged 90 years or older in 2026 is calculated by multiplying the estimated number of patients aged 87 years or older in 2023 by the rate of change in the number of patients aged 87 years or older.

Here, the estimated number of patients aged 87 years or older in 2023 is calculated by adding the estimated number of patients aged 87 to 89 years in 2023 and the estimated number of patients aged 90 years or older in 2023. The estimated number of patients of each age between 87 and 89 years in 2023 is calculated by multiplying the approximate number of acute inpatients aged 84 to 86 years in 2020 who reside in the prefecture by the rate of change for that age, and the estimated number of patients aged 90 years or older in 2023 is calculated by multiplying the approximate number of acute inpatients aged 87 years or older who reside in the prefecture by the rate of change for those aged 87 years or older in 2020. (Note 4) With regard to the "number of inpatients in the convalescent phase by age in the prefecture in 2026" and the "number of inpatients in the chronic phase by age in the prefecture in 2026," the term "acute phase" in Note 3 is

The number is calculated by assuming that these are replaced with "recovery phase" or "chronic phase." (Note 5) Regarding the "Number of hospitalized patients with chronic dementia by age in the relevant prefectures in 2026," "acute" in Note 3 means "cognitive
"Chronic stage of the disease" and "0 to 24 years old" should be changed to "0 to 59 years old".
"And," "25 to 27 years old" is "60 to 62 years old,"
"22 to 24 years old" means "57 to 59 years old" and "28 years old"
"From 63 to 89 years old" should be interpreted as "From 25 to 86 years old" and "From 63 to 89 years old".
"Up to age" means "60 to 86 years old" and "22 to 83 years old."

The number is calculated assuming that the age range is between 57 and 83.

Let us assume that.

(Note 6) The "proportion of policy effect related to chronic inpatients" shall be 0 if the value obtained by dividing the total number calculated by 1) to 4) below by the estimated population of each prefecture and multiplying the result by 1,000 (hereinafter referred to as the "estimated patient rate" in Note 6) is 0.69 or less, or if it exceeds 0.69 and the difference between the estimated patient rate and 0.69 is less than 20% of the estimated patient rate, it shall be 50% of the value obtained by dividing the difference by the estimated patient rate, or if the difference is 20% or more of the estimated patient rate, it shall be 0.1. However, prefectural governors may add or subtract a value between 0 and 0.02 to this calculated ratio, so long as the ratio does not fall below 0.

1) Regarding the estimated number of patients aged 0 to 21 in 2026
are the numbers corresponding to these.

2) The estimated number of patients aged 22 to 24 in 2026 is the sum of the corresponding number and the number obtained by multiplying that number by the rate of change for that age group.

3) The estimated number of patients aged 25 to 86 in 2026 was calculated by multiplying the corresponding number by the rate of change for that age group.

The number is assumed to be 1.

4) The estimated number of patients aged 87 years or older in 2026 was calculated by multiplying the total number of these corresponding numbers by the rate of change in the number of patients aged 87 years or older.

Let it be a number.

* The number of chronic inpatients is based on the perspective of further promoting the shift to the community. Specifically, prefectures whose future estimates of the number of chronic inpatients per capita in 2019, the final year of the 8th Medical Care Plan, exceed the level of prefectures whose number of chronic inpatients per capita in 2020 is sufficiently low, will adjust their future estimates to approach that level.

The coefficient is set.

"Future projections for the number of chronic inpatients per population in 2019"

In calculating the "prefecture-specific population estimates," the "prefecture-specific population estimates" will be based on the future population estimates by region for 2019 published by the National Institute of Population and Social Security Research in "Japan's Future Population Estimates by Region (Estimates for 2018)."

Regarding the "level for prefectures with sufficiently low numbers of chronic inpatients per capita in 2020," we have set the level at 0.69, based on the value actually achieved by the top 10% of the 47 prefectures, as it is necessary to refer to excellent initiatives and set a realistic target.

* The estimated number of patients in each age group from 0 to 24 years old in 2019 is the estimated number of patients in that age group in 2026, based on the idea of 1) in Note 3. The estimated number of patients in each age group from 25 to 27 years old in 2019 is calculated by multiplying the estimated number of patients in each age group from 22 to 24 years old in 2026 by the rate of change of that age group, based on the ideas of 2) and 3) in Note 3. Therefore, when calculating the estimated number of patients in 2019, it is necessary to add up the estimated number of patients in each age group from 22 to 24 years old in 2026 and the value obtained by multiplying that number by the rate of change of that age group.

(Note 7) "Proportion of policy effects on hospitalized chronic dementia patients"

Regarding Note 6, please read as follows: 1) to 4).

The ratio is calculated as follows.

1) "Chronic stage" refers to "chronic dementia."

2) "0.69" becomes "0.34".

3) "0 to 21 years old" is now "0 to 56 years old" and "22 years old and over" is now "From 24 years old to 57 years old" and "From 25 years old to 86 years old" is "between 60 and 86 years old."

4) "Estimated population by prefecture," "Per capita," and "Estimated future population by region in 2010" are the "Estimated population aged 65 and over by prefecture," "Per capita aged 65 and over," and "Estimated future population by region in 2010," respectively.

The report will be titled "Estimated future population of people aged 65 and over by region in Japan."

(Note 8) The "utilization rate of psychiatric beds" is based on the figures used to calculate the standard number of psychiatric beds stipulated in Article 30-4, Paragraph 2, Item 17 of the Medical Care Act (Ministry of Health, Labour and Welfare Notification No. 161 of 2006). (Note 9) The number of inflow and outflow hospitalized patients in each prefecture is determined through patient surveys and National Health Insurance claim surveys, etc.

γ The standard number of tuberculosis beds for each prefecture is decided by the prefectural governor.

The number of beds shall be the

number determined by the prefectural governor. In addition, when calculating the standard number of beds, please refer to

"Calculation of the standard number of beds for tuberculosis in medical care plans" (Notice No. 0719001 of the Director of the Tuberculosis and Infectious Diseases Division, Health Bureau, Ministry of Health, Labour and Welfare, dated July 19, 2005).

γ The standard number of beds for infectious diseases shall be the number determined by the prefectural governor based on the total number of infectious disease beds in specific infectious disease designated medical institutions designated by the Minister of Health, Labour and Welfare pursuant to the provisions of Article 38, Paragraph 1 of the Infectious Diseases Act for each prefecture, and the number of infectious disease beds in first-class infectious disease designated medical institutions and second-class infectious disease designated medical institutions designated by the prefectural governor pursuant to the provisions of Paragraph 2 of the same article. (2) Exceptions to the

calculation of the standard number of beds

If the following circumstances exist when formulating a medical plan, the prefectural governor shall, after hearing the opinion of the prefectural medical council, consult with the Minister of Health, Labour and Welfare and add the number of people who have obtained consent, or shall consult with the Minister of Health, Labour and Welfare and add the number of people who have obtained consent.

The number obtained can be used as the standard number of hospital beds.

γ When a rapid population increase is expected and it is considered necessary to increase the number of hospital beds. γ When there are an unusually large number of people suffering from a particular disease. γ When there are a large number of hospitals capable of providing advanced medical care. γ When there are hospital beds other than those covered by the special provisions for the standard number of hospital beds, due to advances in medicine and medical technology.

When it is considered necessary to have special beds due to

γ Other similar circumstances exist in the area.

In addition, in areas where the elderly population is expected to continue to increase in the future,

A large increase is expected, and it will be necessary to improve the medical care system accordingly.

Therefore, in areas where the number of existing hospital beds exceeds the standard number of beds, if the required number of hospital beds is expected to significantly exceed the existing number of beds in the future,

This will be decided based on the above.

A. Taking into account the increase in medical demand due to the increase in the elderly population, etc.,

Review the plan annually

B. Special measures for calculating the standard number of hospital beds under Article 30-4, Paragraph 9 of

the Law will be implemented. In addition, when developing hospital beds under A and B above, the following points will be taken into consideration and the actual conditions of the region will be fully considered. (A)

Functional classification of hospital beds (functional classification of hospital beds under Article 30-13, Paragraph 1 of the Law)

(a) Trends in medical demand, including after the peak of the elderly

population; (b) Status of medical supply by disease, flow of patients in and out of each medical care area, and transportation facilities.

(D) Distribution of medical institutions in each

medical district within the prefecture, etc.

4. Ensuring consistency with nursing care insurance business (support) plans

Regarding ensuring consistency with the long-term care insurance business (support) plan, the relevant departments will

Specific details will be provided in a notification issued by the Bureau. 5. Steps for establishing a medical cooperation

system for each of the five diseases, six programs, and home medical care

order

(1) Understanding the current situation

In constructing a medical cooperation system, prefectures will collect information on patient trends, medical resources, and medical cooperation, etc., and will take into account the health status of local residents.

It is necessary to understand the current situation.

Among information on the healthcare delivery system, etc., indicators related to the health status of local residents and services that are thought to contribute to improving that status (key indicators), as well as indicators that can be obtained from data provided by other countries, original survey data, and data analysis (reference indicators) will be used to objectively grasp the current state of the regional healthcare delivery system, including the correlations between indicators.

Regarding key indicators and reference indicators, please refer to the attached document "Guidelines for establishing a medical system relating to diseases, projects, and home medical care" in "Regarding medical care systems relating to diseases, projects, and home medical care" (Notification No. 14 of the Director of the Regional Medical Care Division, Medical Affairs Bureau, Ministry of Health, Labour and Welfare, dated March 31, 2023).

In addition, when the current situation is insufficient to grasp from existing statistics and surveys,

It is important to proactively conduct new surveys, such as surveys of facilities and related organizations, questionnaire surveys of patients and residents, and interviews. (Examples of existing statistics and surveys, etc.) 1) Vital Statistics 2) Basic Survey of Living Conditions of the People 3) Patient Survey 4) National Health and Nutrition Survey 5) Examples of Public Health Administration Reports 6) Reports on the Status of Long-Term Care Insurance Programs 7) Hospital Bed Function Reports 8) Receipt Information and Specific Health Checkup Database (NDB) 9) Diagnosis Procedure Combination (DPC) Data 10) Medical Facility Survey 11) Hospital Reports 12) Statistics on Doctors, Dentists, and Pharmacists (formerly: Survey of Doctors, Dentists, and Pharmacists) 13) Reports on Regional Health and Health Promotion Programs 14) Survey of Care Service Facilities and Businesses 15) Statistics on the Status of Long-Term Care Benefits, etc. (formerly: Survey on the Status of Long-Term Care Benefits) (2) Establishment of Working Groups and Regional Cooperation Meetings

In order to build medical systems for the five diseases, six projects, and home medical care, prefectures must establish forums (hereinafter referred to as "working groups") for each of the five diseases, six projects, and home medical care under the prefectural medical council or regional medical care measures council. In addition, as necessary, a forum (hereinafter referred to as "regional cooperation conference") must be established for relevant parties in each region to discuss specific cooperation, etc. It is important that the working groups and regional cooperation conferences work together organically to advance discussions, and in principle, the results of discussions at regional cooperation conferences must be reported to the working groups. In addition, the content and results of each discussion must be reported to the prefectural medical council.

As a general rule, publicize and publicize the information.

ý Working Group

A. Composition

The working group will develop a medical system that is suited to the local conditions.

For example, the organization may consist of representatives of the following (a) to (h), and the category of (a) to (h)

to which the members belong may be clearly indicated.

thing.

(a) Local medical associations and other medical-related

organizations (b) Doctors, dentists, pharmacists, nurses, and other medical practitioners currently engaged in medical practice

Person

(c) Service providers as defined by the Long-Term Care Insurance Act; (d) Health

insurers; (e) Patients and

residents who receive medical and long-term care services; (f) Prefectures and

municipalities responsible for health, medical, and welfare services; (g) Academic experts; (h) In addition

to those listed in (a) to (g) above,

persons who play important roles in each disease and project. Since it is desirable to take fully into consideration the

opinions of those who receive medical services,

efforts should be made to create an environment in which patients and residents in (e) can easily participate in the

discussion by providing them with sufficient information, explanations, and other support.

B. Contents

The Working Group will discuss the following matters: (a) Identifying local

medical resources

Based on information on medical resources and medical cooperation, medical institutions that meet the requirements for each medical function in the region should be identified. Also, taking into account patient trends, etc., medical functions that are lacking or require adjustment or reorganization in the region should be clarified.

Medical resources should be identified based on secondary medical areas as much as possible.

(a) Setting of spheres

Based on the above (A), the sphere will be examined and established. In doing so, important matters specific to each of the five diseases, six businesses, and home medical care will be considered.

(Proportion of inflowing patients for each of the five diseases, six businesses, and home medical care,

Based on the ratio of patients who have left the hospital, we will continue to focus on the traditional secondary medical care area.

Rather, the rates should be set flexibly according to the actual situation of local medical resources, etc.

(c) Identification of issues

Analyze the current situation identified in (a) and identify issues in the regional medical care system for each of the five diseases, six programs, and home medical care, taking into account the direction to be taken, such as the required medical functions and their collaborative systems. In doing so, take into consideration the relevance of the structure, process, and outcome indicators used in the current situation analysis, and based on classification by disease stage and medical functions, extract issues for each medical area as much as possible.

and.

(D) Setting numerical targets

Based on the issues identified, quantitative targets and the time required to achieve the targets will be determined according to the actual conditions of the region so that a quantitative comparative evaluation can be carried out after the fact.

Can.

In setting numerical targets, national data for each indicator will be used as a reference, and targets set out in the plans listed in Basic Policy No. 11 will also be taken into consideration. In addition, targets will not be set that are only achievable, but rather that are believed to truly solve the problems of the medical district.

Set goals that can be achieved.

(E) Measures

A plan will be formulated that includes specific measures to achieve numerical targets corresponding to the issues. In order to solve the identified issues, specific methods will be logically examined, and measures that are as effective as possible will be included. It is also important to show the relationship between each measure and the issues to be solved, so it is considered useful to use tools such as logic models.

The Regional Cooperation Conference is a forum for the parties responsible for each medical function to foster mutual trust, This will be implemented to promote smooth collaboration.

At that time, the public health center will host the conference in cooperation with the local medical association, etc., and Coordinating between institutions or between medical institutions and nursing care service providers, etc. It will play a pivotal role.

A.

Composition All stakeholders in charge of each
medical

function B. Content All stakeholders will share awareness and information on the following (A) to (C).

After having acquired this information, medical institutions will be selected to handle each medical function.

(a) Sharing awareness of the need for medical cooperation; (b)

Sharing information on personnel, facilities, equipment, and medical functions of medical institutions, etc.

Sharing information

(c) Sharing of information on the latest knowledge and medical techniques related to the disease and
project. Also,

depending on the situation, examination of the introduction of regional collaboration critical paths.

To conduct a debate.

(3) Reflecting the opinions of patients and residents

Prefectural governments are required to reflect the opinions of patients and residents by having them
participate in working groups, holding town meetings, conducting interviews and questionnaire surveys of
patients and residents, and publicly commenting on medical plans, and then publish and publicize the
contents of medical plans in an easy-to-understand manner.
be.

(4) Inclusion in medical plan

As shown in Section 3.3, prefectures shall include in their medical plans, for each medical function, the
goals, medical system required of medical institutions, challenges, numerical targets, and measures to be
taken to achieve the numerical targets.

In principle, the names of medical institutions etc. responsible for each medical function should also be listed.
It shall be so.

In some areas, due to limitations on medical resources,
may have multiple functions.

(5) Measures to be taken in the event of changes

If there is a change in the medical institution that will provide medical care after the medical plan is
formulated, the contents of the plan must be changed as soon as possible. In

this case, the change must be made without going through the Prefectural Medical Council each time.

In addition, it is necessary to determine in advance the procedures associated with changing medical institutions.

5. Promotion of medical planning, etc.

1. Medical plan promotion system

Regarding the promotion system for medical planning, it is necessary to build a system in which the relevant parties can share information with each other in working groups, regional cooperation meetings, or regional medical plan coordination meetings, thereby fostering trust and promoting smooth cooperation.

desirable.

2. Understanding, evaluating and reviewing the progress of medical plans

In order to improve the effectiveness of medical plans, it is necessary to set and evaluate specific numerical targets. It is important to carry out an evaluation and then revise the contents of the plan based on the results of that evaluation.

For this reason, as shown in Section 3, paragraph 12, the policy goals, promotion system, promotion measures, evaluation and review methods (including the organization that will carry out the evaluation, such as the Prefectural Medical Council) etc. should be clearly stated in advance in the plan, and the progress of the policy or the medical plan as a whole should be surveyed, analyzed, evaluated and published every six years (every three years for matters related to home medical care, outpatient medical care and securing of doctors), and the plan should be revised if necessary. However, for the five diseases, six projects and home medical care, the

evaluation and review system and publication methods should be clearly stated as above, and the annual trends in numerical targets and the progress of policies should be grasped and evaluated regularly (annually is preferable). If progress toward the goals is insufficient, the cause should be analyzed and the policy should be reviewed as necessary. When conducting the evaluation, it is considered to reuse tools such as logic models used when considering the policies.

can be done.

Section 6 Reports on medical care plans, etc.

In order to contribute to the promotion of appropriate medical plans by grasping the actual conditions of each prefecture and utilizing the information in discussions regarding the revision of these guidelines, etc., the following will be reported to the Minister of Health, Labour and Welfare regarding medical plans pursuant to the provisions of Article 30-4, Paragraph 18 of the Law: shall be treated as follows.

1. Reporting medical plans to the Minister of Health, Labour and Welfare

(1) Reporting matters

• Medical plan and its outline, and attached documents • Method of public notice (attach a copy of the prefectural official notice, website URL, etc.)

ý The date of drafting the plan, the date of hearing opinions from municipalities and insurers' councils, the date of consultation with the prefectural medical council and the date of the report, and the date of public announcement.

ý The results of grasping the current situation through required indicators,

recommended indicators, etc. ý Evaluation of the annual trends in the numerical values

of the target items and the progress of the measures (Note 1) For ý,

submit two copies of the booklet. (Note 2) For ý and ý, submit in electronic form.

(Note 3) For ý, submit once a year from the beginning of the target period of the medical plan.

Putting out

(2) Reporting Period (1) ý

of the medical plan and its outline and accompanying documents shall be reported before the public announcement.

Any other printed matter, if any, shall be reported promptly after publication.

However, in the case of changes to plans that are not based on the provisions of Article 30-6 of the Law, such as changes to the description of medical institutions that will provide medical services in the medical cooperation system for the five diseases, six programs, and home medical care, they will not be subject to reporting. 2 Article 27-2, Paragraph 1, Article 30-11, Article 30-12, Paragraph 2, and Article 30-17 of the Law

(1) Reporting items: Name of hospital or clinic, name of founder

or manager, address of hospital or clinic,

Date of application for opening, number of beds applied for, type of beds applied for, date of request, recommendation

Date, date of order, opinion of the Prefectural Medical Council, response of the applicant to the

recommendation, other reference items, etc.

(2) Reporting period

The report shall be submitted by the 10th of the month following the month in which the request, recommendation or order was made. However, if there is a significant delay in the "response to the recommendation," the relevant part shall be reported at

a later date. In addition, orders based on the provisions of Article 7-2, Paragraph 3 or Article 30-15,

Paragraph 6 of the Law or instructions based on the provisions of Article 30-16, Paragraph 1 of the Law to public medical institutions, e

If any such notice is given, the report must be made in accordance with the attached form.

(Attachment Form 3)

Report on the implementation status of recommendations and orders based on Article 27-2 of the Medical Care Act

Hospital or clinic name			
Name of creator or administrator			
Hospital or clinic location			
Number of hospital beds applied for (bed function type)		Number of hospital beds permitted under conditional payment (bed function type)	
Recommendation Date			
Opinion of the Prefectural Medical Council			
Response to the Recommendation			
Order date			
Opinion of the Prefectural Medical Council			
Reaction to orders			
Other reference points			

(Attachment Form 4)

Report on the implementation status of recommendations based on Article 30-11 of the Medical Care Act

Hospital or clinic name			
Name of creator or administrator			
Hospital or clinic location			
Year, month and day of application for opening etc.			
Number of hospital beds requested		Type of hospital bed applied for	
Recommendation Date			
Opinion of the Prefectural Medical Council			
Applicant's response to the recommendations			
Other reference points			

(Attachment Form 5)

Report on the implementation status of requests based on the provisions of Article 7-2, Paragraph 3, as applied mutatis mutandis pursuant to the provisions of Article 30-12, Paragraph 1 of the Medical Care Act, and recommendations based on the provisions of Paragraph 2 of the same Article

Hospital or clinic name			
Name of creator or administrator			
Hospital or clinic location			
Date of reduction request		Request for reduction in number of hospital beds (by type of hospital bed)	
Opinion of the Prefectural Medical Council			
Response to the request			
Recommendation Date		Number of recommended hospital beds (by type of hospital bed)	
Opinion of the Prefectural Medical Council			
Response to the Recommendation			
Other reference points			

(Attachment Form 6-1)

Report on the implementation status of requests based on the provisions of Article 30-15, Paragraph 6 of the Medical Care Act, as applied mutatis mutandis pursuant to the provisions of Article 30-15, Paragraph 7 of the same Act, and recommendations based on the provisions of Article 30-17 of the same Act

Hospital or clinic name			
Name of creator or administrator			
Hospital or clinic location			
Reference date bed function		Hospital bed performance after base date	
Date of request			
Opinion of the Prefectural Medical Council			
Response to the request			
Recommendation Date			
Opinion of the Prefectural Medical Council			
Response to the Recommendation			
Other reference points			

(Attachment Form 6-2)

Report on the implementation status of requests based on the provisions of Article 30-16, Paragraph 2 of the Medical Care Act, as applied mutatis mutandis, and recommendations based on the provisions of Article 30-17 of the Medical Care Act

Hospital or clinic name			
Name of creator or administrator			
Hospital or clinic location			
Conversion request date			
Hospital bed function requested to be converted		Number of hospital beds requested for conversion	
Opinion of the Prefectural Medical Council			
Response to the request			
Recommendation Date			
Opinion of the Prefectural Medical Council			
Response to the Recommendation			
Other reference points			